The Menopause Diet for Busy Women

Easy Recipes, a 12-Week Meal Plan, and Simple Daily Practices to Support Weight Management, Energy, and Wellbeing

[Author Name]

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Recipes have been written with care, but individual tolerances vary. Please check every ingredient against your own allergies and intolerances.

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A Letter to You

Dear reader,

If you've picked up this book, there's a good chance something has changed. Maybe you're waking at 3 a.m. for no obvious reason. Maybe your periods have started to behave like they have a mind of their own, or have stopped altogether. Maybe the jeans that fitted perfectly last spring now sit a little tighter around the middle, even though you haven't changed a thing. Maybe you've cried in the car, snapped at someone you love, or walked into a room and forgotten why you were there, and wondered quietly whether something is wrong with you.

Let me say this as clearly as I can: **you are not broken. You are changing.**

Why I Wrote This Book

I was forty-four the first time I woke up soaked through, heart racing, convinced I was coming down with something. I wasn't. It happened again the next night, and the one after that. Within a few months I was sleeping in four-hour scraps, forgetting the names of people I'd known for years, and crying at commercials. I put it all down to stress. I had a full-time job, two teenagers and a mother who needed more help every month. Of course I was tired.

When I finally told my doctor, she ran some blood tests, said everything looked "normal," and suggested I try to relax. I left that appointment feeling more alone than when I'd walked in.

It took me almost two years, a lot of late-night reading and a second doctor who actually listened before I understood what was happening: I was in perimenopause, and nobody had thought to mention it. Not my doctor, not my friends, not my own mother, who, when I finally asked her, said, "Oh, that. We just got on with it."

I don't want you to just get on with it. Somewhere along the way I started changing what I ate, not to shrink myself, but to feel steadier. More protein

at breakfast. More plants. Fewer late glasses of wine. I started lifting weights in my living room. None of it was dramatic, but together it changed how my days felt. The night sweats eased. The 3 p.m. crash stopped running my afternoons. I felt like myself again, a stronger, calmer version.

This is the book I wish someone had handed me at forty-four.

What You'll Find Here

Menopause is a natural stage of life that every woman who lives long enough will reach. Yet for generations it's been whispered about, joked about or simply ignored. If you've felt confused, dismissed or alone, that isn't a personal failing. It's a gap in the information we've been given.

In the pages ahead you'll find a clear, kind explanation of what's happening in your body during perimenopause, menopause and the years beyond. You'll learn how food can support your energy, sleep, mood and comfort, and how to care for your bones, heart, muscles and brain for the long haul. And you'll find tools built for the busiest of lives: quick recipes, snacks you can make in five minutes, a 12-week meal plan with grocery lists, and a 30-day plan of yoga, strength and calming practices that needs nothing more than a little floor space.

Here's what this book will never do. It will never shame you: there are no "good" or "bad" foods here, no punishing rules and no before-and-after pressure, because your worth has nothing to do with the number on a scale. It will never promise miracles: food is powerful, but it isn't magic, and some symptoms need medical support, which is perfectly okay. And it will never ask you to change everything at once. One breakfast is a fine place to start.

You deserve to feel strong, comfortable and like yourself again, and you deserve to enjoy the food on your plate while you get there. Wherever you are in this transition, this is a wonderful time to start looking after yourself with the same care you've probably given everyone else for years.

Make a cup of tea, find a comfortable chair, and let's begin.

With warmth,

[Author Name]

How to Use This Book

This book is meant to be used, not just read. Dip in wherever you need to, on whatever kind of day you're having. There's no single right way through it, but there are three easy ways in.

Three Ways In

Read it cover to cover. If you like to understand the "why" before the "how," start with Part One, which walks through each stage of the transition in plain language. Part Two covers treatments, therapies and the everyday habits with the strongest evidence behind them. Part Three turns all of it into food.

Go straight to what's bothering you. If hot flashes, broken sleep or afternoon crashes are your biggest problem right now, turn to Chapter 10, "Eating for Your Symptoms." Each symptom has its own section, with what may help, what may make it worse and one or two things to try this week. There's also a symptom index at the back.

Start cooking tonight. Read Chapter 8, "Not Sure of Your Stage? Start Here," and Chapter 12, "The Busy Woman's Kitchen," then turn to the 12-week meal plan in Part Five. You can come back to the background chapters whenever you're curious.

The One Idea to Remember: Anchor, Add, Adapt

If you only take one thing from this book, take these three words. They're the method behind every chapter, recipe and meal plan in it.

- **Anchor** every meal with protein, about 25 to 30 grams. It's the nutrient midlife muscles need most and the one most of us eat least of in the morning.
- **Add** plants: vegetables, fruit, beans, whole grains, nuts and seeds, aiming for 30 different ones a week.
- **Adapt** to where you are: your stage of the transition, your symptoms, your appetite and your life this week.

You'll meet Anchor, Add, Adapt properly in Chapter 9, and you'll see it at work in every recipe.

How the Book Fits Together

Part	What you'll find	Best for
Part One	What's happening in your body and mind at each stage	Understanding and reassurance
Part Two	Treatments, therapies and everyday habits	Informed conversations with your doctor
Part Three	Anchor, Add, Adapt; eating for your symptoms; weight; kitchen skills	Daily eating decisions
Part Four	30 main recipes and 25 simple snacks	Cooking
Part Five	The 12-week meal plan with grocery lists and prep plans	Structure and routine
Part Six	The 30-day yoga, strength and calm plan	Movement, stress and sleep

Along the way you'll see a few recurring boxes. **Try This Week** suggests one or two specific things to test. **What the Evidence Says** tells you where the science is strong and where it's still emerging. **Good to Know** adds practical tips and safety notes. Some chapters end with a checklist you can photograph or print.

Recipe Labels

Label	Meaning
GF	Gluten-free (or "GF option" with a simple swap)
DF	Dairy-free (or "DF option")
V	Vegetarian
VG	Vegan (or "VG option")
Make-Ahead	Keeps well for batch cooking or freezing
30 min	Ready in 30 minutes or less (the quickest show their exact time, e.g., "10 min")

Take It One Change at a Time

You don't need to overhaul your life this week. In fact, it works better if you don't. Pick one change that feels doable, like adding protein to breakfast or drinking a glass of water before your morning coffee, and let it settle into routine before you add the next.

> **KEEP YOUR DOCTOR IN THE LOOP**
>
> This book offers general, evidence-informed guidance, but it can't know your personal health history. If you have a medical condition, take regular medication, are thinking about hormone therapy or supplements, or have symptoms that worry you, talk to your doctor or another qualified healthcare professional. A good clinician is one of the best allies you can have during this transition.

A Note on the Science

Menopause research has grown enormously in the past twenty years, and it's still moving fast. Some recommendations rest on large, well-designed studies that have been repeated many times. Others are based on promising early findings that need more work before anyone can be confident. A few popular claims have very little evidence behind them at all.

Throughout this book I've tried to be honest about which is which. Where it matters, you'll see two labels. **Established** means the advice is backed by strong, consistent evidence, such as large clinical trials, systematic reviews or the guidelines of major health organizations, and you can act on it with confidence. **Emerging** means the research is early or mixed: interesting, and possibly worth trying if it's safe for you, but not settled.

Where the Information Comes From

The guidance here draws on the published recommendations of The Menopause Society (formerly the North American Menopause Society), the UK's National Institute for Health and Care Excellence (NICE), the National Health Service (NHS), the US National Institutes of Health (NIH), the World Health Organization (WHO) and leading bone, heart and cancer research organizations, as well as peer-reviewed studies. You'll find references and trusted resources at the back of the book.

Science deals in averages, and you're an individual. A change that helps most women may do nothing for you, and something with only emerging evidence may make a real difference in your life. Stay curious, try one change at a time, notice how you feel, and talk to a healthcare professional about anything that affects your medical care. If something here differs from what your doctor tells you, trust the person who knows your history, and feel free to bring this book to your next appointment.

About the Women in These Pages

Each chapter opens with a short story about a woman going through the transition. These women are composites. Their stories are drawn from experiences that are common in research, in support groups and in conversations with friends, but the names, ages and details are invented, and none of them describes a real, identifiable person. They come from different backgrounds, cultures and stages of life, because menopause looks different for every woman, and I wanted more of you to recognize yourselves here.

PART ONE

Understanding Your Changing Body

Before we talk about food, let's talk about you: what's happening, why it's happening, and why none of it is your fault.

CHAPTER 1

The Menopause Transition, Explained Simply

Menopause lasts exactly one day, and almost no woman knows which day it was until a year later.

> *Renée was forty-seven when she sat in her car outside the school where she taught, trying to work out what was wrong with her. Her periods had gone from clockwork to chaos: three weeks apart, then seven, then a flood that soaked through everything. She'd started waking at two every morning. Her doctor had said "probably stress" and handed her a leaflet on sleep hygiene. Her sister said she was too young for "the change." Her best friend, a year older, swore she hadn't noticed a thing. Renée turned off the engine and typed "am I going crazy or is this menopause" into her phone. Then she deleted it, embarrassed, and went inside to teach second period.*

Renée isn't going crazy. She's in the middle of one of the least explained transitions in a woman's life, and like most of us, she was never given a map.

For something that happens to half the world's population, menopause is remarkably misunderstood. Most of us grew up picturing a single moment, the day periods stop, somewhere around fifty. In reality the transition unfolds over years, sometimes more than a decade, and it often begins long before we expect. This chapter is the map. Once you know what your hormones are doing and why, everything else in this book will make more sense, and a lot of the confusing things you've been feeling will finally have an explanation.

One Day, Many Years

Strictly speaking, **menopause is a single point in time**: the day that marks twelve months since your last period. You can only spot it looking backward.

Everything before it is **perimenopause**, and everything after it is **post-menopause**. In everyday conversation, most of us use "menopause" for the whole journey, and that's fine. In this book I'll use the specific terms when the difference matters and the general one when it doesn't.

It helps to picture your reproductive life as a long road. For most of your adult years it's fairly smooth: hormones rise and fall in a predictable monthly rhythm. Perimenopause is the stretch where the road turns bumpy and unpredictable. Menopause is the signpost you pass after a full year without a period. Post-menopause is the new, smoother road beyond it. The bumpy section is where most women feel the most symptoms, and it lasts around four years on average, although for some women it's much shorter and for others it runs ten years or more.

When It Arrives, and How

In the United States, the average age of natural menopause is 51 to 52, and most women get there somewhere between 45 and 55. Your genes play a part, so women often reach menopause at a similar age to their mothers and sisters, and smoking tends to bring it forward by a year or two.

Not every menopause arrives gradually, though. In a **natural menopause**, the ovaries slowly wind down their production of hormones over several years. A **surgical menopause** happens overnight when both ovaries are removed, and because hormone levels drop suddenly rather than gently, symptoms can be abrupt and intense. A **medically induced menopause** can follow some cancer treatments, such as chemotherapy or pelvic radiation, or medicines that deliberately quiet the ovaries, and it may be temporary or permanent.

A hysterectomy that removes the uterus but leaves the ovaries in place ends periods, but it doesn't cause menopause on its own: the ovaries keep working, although some women reach menopause a little earlier afterward. Without periods to go by, it's simply harder to tell when the transition starts, so symptoms become your main clue.

If Your Timeline Is Different

Around 1 in 100 women reach menopause before the age of 40, which is called **premature ovarian insufficiency (POI)**. About 5 in 100 more reach it between 40 and 45, known as **early menopause**.

If that's you, your experience deserves particular care. Reaching menopause early means living longer without estrogen's protective effects on your bones, heart and brain, which is why guidelines generally recommend that women with POI or early menopause talk to a doctor about hormone therapy, usually at least until the average age of natural menopause, unless there's a medical reason not to. That's a very different conversation from the one a woman has at 55 or 60. An early or sudden menopause can also bring grief, for fertility, for the timeline you imagined, or simply for feeling "too young" for this. Those feelings are valid, and support groups for women with POI can be a lifeline.

> **WHEN TO SEE A DOCTOR SOONER**
>
> If you're under 45 and your periods have become irregular or stopped, or you're having symptoms such as hot flashes and night sweats, see your doctor. Early menopause is diagnosed differently from menopause at the usual age, and it often benefits from specific treatment.

The Cast of Characters: Your Hormones

Hormones are chemical messengers. They travel through your bloodstream telling different parts of your body what to do, and during the transition several of them change at once. Because they reach almost every system in the body, the effects can show up in surprising places.

Estrogen is the star of the show. Made mainly by the ovaries during your reproductive years, it does far more than run your cycle. It slows the rate at which old bone is broken down, keeps blood vessels flexible and cholesterol in a healthier range, helps your brain regulate temperature, supports mood, memory and sleep, keeps the skin and the tissues of the vagina and bladder plump and moist, and influences where your body stores fat. After menopause the ovaries make very little of it. Your body still produces small amounts in fat tissue and elsewhere, but levels are much lower than before.

Progesterone is produced after you ovulate each month. It prepares the lining of the uterus for a possible pregnancy and has a naturally calming, sleep-friendly effect on the brain. In perimenopause, ovulation becomes less regular, so progesterone is often the first hormone to fall, which can mean heavier or irregular periods, lighter sleep and feeling more on edge.

Testosterone isn't just a male hormone. Women make it too, in the ovaries and adrenal glands, and it contributes to libido, energy and muscle. It declines gradually from your twenties onward rather than dropping at menopause.

FSH (follicle-stimulating hormone) comes from the pituitary gland in your brain, and its job is to tell the ovaries to ripen an egg each month. As the ovaries become less responsive, the brain shouts louder, pumping out more FSH to try to get a reply. That's why FSH tends to rise during the transition. But in perimenopause it can swing up and down from month to month, which is why a single blood test so often tells you very little (more on that in Chapter 3).

Cortisol isn't a reproductive hormone, but it's always in the background. Released in response to stress, it helps regulate blood sugar, blood pressure and your sleep–wake cycle. Broken sleep, night sweats and the pressures of midlife can keep it running high, which can affect appetite, belly fat and mood. The encouraging part is that cortisol responds well to everyday habits, like regular meals, movement, rest and calming practices, and you'll find all of those in this book.

It's the Swings, Not Just the Drop

It's tempting to imagine perimenopause as a slow, steady slide in hormones. In reality it's more like a rollercoaster. Some months, estrogen can surge higher than it was in your thirties; the next month it plunges. You might ovulate one month and not the next.

Those sharp ups and downs are behind many of the most maddening symptoms of perimenopause: periods you can't predict, sore breasts, migraines, mood swings and sleep that's fine one week and terrible the next. Many women find that once they reach post-menopause and hormones settle

at a new, steady low, some of these symptoms ease, even if others, like vaginal dryness, carry on.

So if your symptoms seem to come and go with no pattern, you haven't imagined it, and you aren't doing anything wrong. Your body is adjusting to a changing landscape.

The Three Stages at a Glance

	Perimenopause	Menopause	Post-menopause
What it is	The transition years before the final period	The point 12 months after your last period	All the years that follow
Typical timing	Often begins in the 40s; lasts around 4 years on average, sometimes 10 or more	Average age 51 to 52	From menopause onward
Hormones	Fluctuating, sometimes wildly	Estrogen and progesterone now low	Settled at a new, low level
Periods	Irregular, heavier, lighter or skipped	None for 12 months	None (any bleeding needs checking)
Common focus	Cycle changes, sleep, mood, hot flashes, energy	Hot flashes, sleep, joints, intimacy, body composition	Bone, heart, muscle and brain health; lingering symptoms

> **BLEEDING AFTER MENOPAUSE**
>
> Any vaginal bleeding after you've gone 12 months without a period should be checked by a doctor promptly. It's often caused by something harmless, but it always needs investigating.

Why Your Menopause Won't Look Like Your Friend's

Some women sail through with barely a hot flash. Others find their lives turned upside down. Most land somewhere in between. Around three in four women have hot flashes and night sweats at some point, and roughly a quarter find them severe enough to disrupt daily life.

What shapes your experience? Genes are part of it, which is why your mother's age at menopause is a reasonable clue to your own. Your health history matters too: thyroid disease, diabetes, migraines or a history of depression can all change how the transition feels. Smoking, heavy drinking, very little activity and poor sleep tend to make symptoms harder to manage, and women with a higher body weight often report more hot flashes in perimenopause. Then there's life itself. Midlife often brings children, aging parents, career pressure and relationship changes all at the same time as hormonal change. Stress doesn't cause menopause, but it can turn the volume up on everything you feel. Even attitudes matter: research suggests that the way a culture views aging and menopause can influence how women experience it.

Culture, Ethnicity and Your Menopause

One of the largest long-term studies of women in midlife, the Study of Women's Health Across the Nation (SWAN), followed thousands of American women of different ethnic backgrounds for more than two decades, and it found real differences between groups. On average, Black women reported hot flashes for longer, often around ten years, than white women, while women of Japanese and Chinese heritage reported fewer hot flashes overall. Black and Hispanic women also tended to reach menopause slightly earlier.

These are averages, not predictions, and the reasons are complex: genetics, diet, stress, access to healthcare and the effects of discrimination all play a part. What matters for you is this: **your experience is valid, even if it doesn't look like what you've read about or what your friends describe.** If you feel your symptoms are being brushed off, Chapter 6 has practical tips for advocating for yourself at appointments.

Food traditions matter too. The recipes in this book draw on Mediterranean, South Asian, East Asian, Latin American and West African kitchens, because nourishing food looks different all over the world, and there's no single "right" menopause diet.

CHAPTER SUMMARY

- Menopause is technically one day: the point 12 months after your final period. Perimenopause comes before it and post-menopause after it.
- The average age of natural menopause is 51 to 52. Menopause before 45 is early, and before 40 it's called premature ovarian insufficiency; both deserve medical attention.
- Estrogen affects your bones, heart, brain, temperature, mood, skin and fat storage, which is why symptoms can show up almost anywhere.
- In perimenopause it's the wild swings in hormones, not just the decline, that drive many symptoms.
- Genetics, health, lifestyle, stress and culture all shape your transition.

• • •

So if the road turns bumpy in your forties, how do you know you've reached it? That's where we're going next.

CHAPTER 2

Perimenopause: The Shifting Years

Perimenopause can begin up to ten years before your final period, often while you're still being told you're too young for it.

> *Priya was forty-four and had a spreadsheet for everything: her team's deadlines, her son's soccer schedule, the family's grocery budget. What she couldn't organize was her body. Her period now arrived every twenty-two days, heavier each time. She woke at 3 a.m. with her T-shirt stuck to her back, then lay awake running through tomorrow's meetings. By ten each morning she'd hit a wall so hard she kept a stash of cookies in her desk drawer. When she mentioned the night sweats to a colleague, the answer came back with a laugh: "You're way too young for that." Priya laughed too. Then she went home and added a new tab to her spreadsheet: symptoms.*

If you've been told you're too young for menopause while lying awake drenched in sweat, this chapter is for you.

Perimenopause is the part of the transition women know least about, and it's often the stage with the most noticeable symptoms. It usually begins in your forties, though for some women it starts in their late thirties. Because periods are still happening, many women (and, frustratingly, some healthcare providers) don't connect new symptoms with hormones at all. That was my experience too. My first doctor looked at a normal-looking blood test and told me I was fine. I wasn't fine. I was perimenopausal.

The First Signs Are Easy to Miss

There's no single test that tells you perimenopause has started. It's usually recognized by a pattern, and the earliest changes are easy to blame on something else.

Your cycle is often the first thing to shift. Periods might start coming closer together, every 21 to 24 days instead of every 28, or further apart, with the odd skipped month. Bleeding can become heavier, lighter, longer or shorter than you're used to, and premenstrual symptoms may get worse. A change in cycle length of seven days or more that sticks around for several months is one of the classic early markers. Later in the transition, gaps of 60 days or more become common.

Sleep often changes before anything else. You might find it harder to drift off, wake more often during the night, or wake very early and be unable to get back to sleep. Night sweats can play a part, but sleep can also become lighter without them, partly because progesterone, that natural calmer, is falling.

New or worsening anxiety, irritability and low mood are also very common, and they're often among the very first signs. Chapter 5 is devoted to them.

Heavy bleeding deserves a special mention. When estrogen swings high without regular progesterone to balance it, the lining of the uterus can build up more than usual, which can mean heavier periods or even flooding. Other women find their periods become lighter and shorter. Both can be normal, but very heavy bleeding should always be checked (see the end of this chapter).

Hot, Achy, Bloated: What Else Can Change

As the transition goes on, other symptoms often join in. You might have some, many or very few of them.

- **Hot flashes and night sweats:** a sudden wave of heat through the face, neck and chest, sometimes with flushing, sweating, a racing heart and then chills. At night they're called night sweats, and doctors call both *vasomotor symptoms*.
- **Breast tenderness,** from estrogen peaks that are higher than usual.
- **Headaches and migraines,** especially if you've had hormone-related migraines before. They often improve after menopause.
- **Bloating and digestive changes,** because hormones affect fluid balance and the gut.
- **Joint aches,** particularly stiffness first thing in the morning.
- **Weight and shape changes,** often around the middle, even without changes in what you eat. Chapter 11 explains why.

Why your internal thermostat goes haywire

Your brain has a thermostat in an area called the hypothalamus, and estrogen helps keep it stable. As estrogen swings and falls, the comfortable temperature zone narrows, so even a tiny rise in core temperature can set off the full cooling response: blood vessels near the skin widen, you flush and sweat, and your heart speeds up. Researchers have pinpointed a group of brain cells, called KNDy neurons, that become overactive when estrogen is low. That discovery has led to a new class of non-hormonal treatments, which you'll meet in Chapter 6.

The Symptoms Nobody Warned You About

Estrogen has receptors throughout the body, so perimenopause can show up in ways most women have never heard of. Some feel a fluttering, pounding or racing heartbeat, sometimes during a hot flash. Usually it's harmless, but palpitations should always be checked by a doctor, especially if you also feel dizzy or breathless or have chest pain. Others notice itchy, dry skin, or even a crawling sensation like insects on the skin, which has its own name: *formication*. Ringing in the ears (tinnitus) is reported by some women during the transition, and frozen shoulder, a painful stiffness that limits movement, is most common in women between 40 and 60, with hormonal change a likely factor. Dry eyes, a dry mouth, tingling in the hands or feet, a burning feeling in the mouth and new allergies can all appear too.

None of these means something is seriously wrong. Each one deserves to be taken seriously and checked.

The 10 a.m. Crash and the Blood-Sugar Connection

Many women describe a new kind of tiredness in perimenopause: not just sleepy, but drained, foggy and flat. Broken sleep is the obvious culprit. Heavy periods can lower your iron. Stress keeps your body on high alert. And changing estrogen levels can affect how sensitive your body is to insulin, the hormone that moves sugar from your blood into your cells.

When your body becomes a little less sensitive to insulin, a breakfast of toast and jam, a pastry or a sweetened coffee can send your blood sugar up quickly and then down just as fast. That dip shows up as an energy crash, irritability ("hangry") and a powerful craving for something sweet, which sets the whole

cycle going again. Priya's 10 a.m. cookie stash wasn't a lack of discipline. It was her body asking for fuel.

This is one of the places where food can make the biggest difference. Meals that combine protein, fiber and healthy fats are digested more slowly and keep your energy steady. It's the heart of the Anchor, Add, Adapt method in Chapter 9.

What to Put on Your Plate Now

Perimenopause is the time to start building habits that will serve you for decades, and a few nutrients deserve extra attention.

Nutrient	Why it matters in perimenopause	Good food sources
Iron	Heavy periods can drain iron, leading to fatigue, breathlessness and hair shedding	Lean red meat, poultry, fish, lentils, beans, tofu, spinach, fortified cereals
B vitamins	Support energy production and the nervous system	Whole grains, eggs, dairy, legumes, leafy greens, meat and fish
Magnesium	Involved in muscle relaxation, sleep and blood-sugar control	Pumpkin seeds, almonds, cashews, spinach, black beans, whole grains, dark chocolate
Protein	Keeps you full, steadies blood sugar and protects muscle	Eggs, Greek yogurt, fish, poultry, tofu, tempeh, beans, lentils
Fiber	Supports digestion, fullness and steady energy	Vegetables, fruit, oats, beans, lentils, whole grains, seeds

If your periods are heavy, ask your doctor to check your iron levels, including ferritin, which measures your iron stores. Low iron is very common in perimenopause and easy to miss. Pairing iron-rich plant foods with vitamin C, from peppers, tomatoes, citrus or berries, helps your body absorb more of it, while tea and coffee reduce absorption, so drink them between meals rather than with them. Please don't start iron supplements without checking first: too much iron can be harmful.

Is It Perimenopause, or Something Else?

Many perimenopause symptoms overlap with other common conditions. That's why it matters not to assume everything is "just hormones," and equally not to accept that everything is "just stress."

An underactive thyroid is common in midlife women and can cause tiredness, weight gain, low mood, dry skin, thinning hair and changes to periods, while an overactive one can cause anxiety, palpitations, sweating and poor sleep. A simple blood test checks both. Low iron or anemia brings fatigue, breathlessness, dizziness and feeling cold. Low vitamin D or B12 can cause tiredness, low mood and aches. Diabetes can show up as tiredness, thirst and needing to urinate often, and sleep apnea, which becomes more common after menopause, causes exhaustion, morning headaches and poor concentration. Depression and anxiety disorders can appear at any age, and the risk of depression rises in perimenopause, especially if you've had depression, postnatal depression or severe premenstrual symptoms before.

It's entirely possible to be in perimenopause *and* have one of these at the same time. A good doctor will consider both.

When to Pick Up the Phone

You don't need to wait until your symptoms are unbearable. It's always reasonable to see a healthcare professional if your symptoms are affecting your life, and in particular if you notice:

- ☐ Very heavy bleeding (soaking a pad or tampon every hour or two, passing large clots, or bleeding for more than seven days)
- ☐ Bleeding between periods or after sex
- ☐ Periods more often than every 21 days
- ☐ Symptoms of perimenopause before the age of 45
- ☐ Palpitations, especially with dizziness, chest pain or breathlessness
- ☐ Low mood, anxiety or thoughts of self-harm (seek help urgently)
- ☐ Symptoms disrupting your sleep, work or relationships

For women over 45 with typical symptoms, perimenopause is usually diagnosed from your symptoms and cycle pattern alone. Blood tests generally aren't needed and can mislead, because hormone levels change so much from

day to day. They can be useful in specific situations, for example if you're under 45 or your doctor wants to rule out something else.

TRY THIS WEEK

- Start a symptom diary, like Priya's spreadsheet. Each evening, note your cycle day, sleep, energy, mood and any hot flashes. After four to six weeks patterns usually appear, and your doctor's appointment will be far more productive. (Appendix A has a tracker.)
- Anchor your breakfast with a palm-sized portion of protein on at least four days this week.

CHAPTER SUMMARY

- Perimenopause usually begins in your forties and lasts around four years on average, though it can be shorter or much longer.
- Early signs include cycle changes, disrupted sleep and new anxiety or low mood.
- Hot flashes happen because the brain's comfortable temperature zone narrows as estrogen fluctuates.
- Palpitations, itchy skin, tinnitus and frozen shoulder are real perimenopause symptoms and worth discussing with a doctor.
- Meals anchored with protein, fiber and healthy fats protect your energy and calm cravings.
- Thyroid problems, low iron and depression can mimic perimenopause, so it's worth checking.

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Eventually the bumpy years lead to a quiet milestone you'll only recognize in hindsight. Next: what happens when the periods stop.

CHAPTER 3

Menopause: Reaching the Milestone

Hot flashes last more than seven years on average. Most women are told to expect a few months.

> *Carmen was halfway through presenting the quarterly numbers when it started: a slow burn at the base of her neck that rose to her face in seconds. She felt her blouse dampen. Twelve faces looked at her slides while she looked for the nearest window. Later, in the restroom, holding a paper towel under the cold tap, she did the math. Her last period had been the week of her daughter's graduation, more than a year ago. She was fifty-one. She'd assumed that when periods stopped, the rest would stop too. Instead, the hot flashes seemed to be getting stronger. "So this is it," she told her reflection. "This is menopause." Then she pressed the cold towel to her neck and went back in to finish her slides.*

Menopause, the day you've gone twelve months in a row without a period, passes quietly. There's no ceremony, and you only know you've reached it looking back. Yet the years around this point are often when hot flashes and sleep problems are at their strongest, and when longer-term changes to metabolism, bones and heart begin to speed up. This chapter is about what happens around the milestone and how to look after yourself through it.

How You'll Know You're There

For most women over 45, menopause is confirmed simply by looking back: if you haven't had a period for twelve months, and there's no other explanation such as pregnancy, certain medicines or a medical condition, you've reached menopause.

Why a blood test often can't tell you

Many women expect a blood test to give them a clear answer. In practice, guidelines such as NICE's say that FSH tests usually *aren't* needed to diagnose perimenopause or menopause in women over 45 with typical symptoms. Hormones swing so much during the transition that a single result can be misleading: one month your FSH looks "menopausal," the next it's back in the normal range.

Blood tests do earn their place in some situations: if you're under 45 and your doctor suspects early menopause or premature ovarian insufficiency, if you use hormonal contraception (like an IUD) that stops your periods and hides where you are in the transition, or if your doctor wants to rule out other causes, such as thyroid disease.

If you use hormonal contraception, ask your doctor when and how it's safe to stop, because you can still get pregnant in perimenopause. Contraception is generally advised until a year after your last period if you're over 50, or two years if you're under 50.

The Symptoms That Can Peak Now

For many women, the year or two either side of the final period is when hot flashes and night sweats are most frequent and intense. The SWAN study found that hot flashes last a median of about seven years in total, and more than ten for some women. That's far longer than the "few months" many of us are told to expect, so if yours are lingering, you're in very good company, and you don't have to simply wait them out. There are effective treatments, which Chapter 6 covers.

Sleep often suffers at the same time. Night sweats wake you, anxiety keeps you up, and you may need the bathroom more often. Over weeks and months, poor sleep affects your mood, appetite, concentration and resilience. Chapters 7 and 10 have practical strategies for better nights, and the evening routines in Part Six are designed to help you wind down.

Joint pain and stiffness are common around menopause too, often in the hands, knees, hips and shoulders. Estrogen has anti-inflammatory effects and helps maintain cartilage and the fluid that cushions your joints, so as it

falls, joints can start to ache. Regular movement, strength work and an anti-inflammatory way of eating (Chapter 9) all help.

Desire, Comfort and Connection

This is the topic many women find hardest to raise, even with a doctor. Yet changes to sexual health are among the most common effects of menopause, and unlike hot flashes, they often don't improve on their own. You deserve to feel comfortable in your body, and there are treatments that work.

As estrogen falls, the tissues of the vulva, vagina and bladder become thinner, drier and less elastic. Doctors call this **genitourinary syndrome of menopause (GSM)**, and it affects around half of post-menopausal women, possibly more. It can mean dryness, itching or burning, discomfort or pain during sex, needing to urinate more often or more urgently, and more frequent urinary tract infections.

Desire can change too. Lower hormones are part of it, but so are tiredness, stress, body image, relationship dynamics and simply not feeling like yourself. Low libido is common and nothing to be ashamed of.

What helps? Vaginal moisturizers used two or three times a week can relieve everyday dryness, and lubricants make sex more comfortable; water-based and silicone-based options are easy to find. Low-dose vaginal estrogen, as a cream, tablet or ring, is very effective for GSM and is absorbed into the bloodstream only in tiny amounts (more in Chapter 6). Pelvic floor physical therapy can help with pain, tension and bladder symptoms. And honest conversation, with a partner and with your doctor, can make an enormous difference.

Your Metabolism Hasn't Broken

Many women feel their body starts "working differently" around menopause. You eat the same foods and move the same amount, yet the scale creeps up or your shape changes.

Here's something that surprises most people: **your metabolism barely changes between the ages of 20 and 60.** A landmark study published in *Science* in 2021, which measured the daily energy use of more than 6,000 people from infancy to old age, found that once you account for body size,

metabolic rate stays remarkably steady through adulthood and only begins a slow decline after 60.

So what's going on? Mostly, it's your muscle. Muscle mass naturally declines with age, and the loss tends to speed up around menopause. With less estrogen, your body also tends to store more fat around the abdomen rather than on the hips and thighs. Insulin sensitivity can dip, making blood sugar harder to steady, and poor sleep and stress push up hunger hormones and cravings for quick energy.

That's genuinely good news, because muscle, activity and sleep are things you can influence. Building and protecting muscle, through strength training and enough protein, is one of the most effective things you can do for your metabolism, your blood sugar and your long-term health. Chapter 11 goes deeper, with no crash diets in sight.

Skin, Hair and Nails

Estrogen supports collagen, the protein that gives skin its structure and bounce, and skin can lose up to a third of its collagen in the first five years after menopause. You may notice drier, thinner, more sensitive skin that's slower to heal, and fine lines appearing faster. Hair changes too: some women see thinning on the scalp, others new hairs on the chin or upper lip as the balance between estrogen and testosterone shifts. Nails may become more brittle.

Daily broad-spectrum sunscreen is the most effective anti-aging step there is, and generous moisturizing after bathing, with fragrance-free products if your skin is sensitive, helps a lot. From the inside, enough protein gives your body the building blocks for skin, hair and nails; olive oil, avocado, nuts, seeds and oily fish support the skin's natural barrier; and colorful fruit and vegetables provide vitamin C, which your body needs to make collagen. Drink enough water, and if you smoke, stopping slows skin aging dramatically. Significant hair loss is worth raising with your doctor, since low iron and thyroid problems are common, testable causes.

What to Put on Your Plate Now

Nutrient	Why it matters now	Good food sources
Protein	Protects muscle as it gets harder to maintain; supports skin, hair and nails	Fish, poultry, eggs, Greek yogurt, cottage cheese, tofu, tempeh, edamame, beans, lentils
Calcium	Bone loss speeds up in the years around menopause	Dairy, calcium-set tofu, fortified plant milks, canned fish with bones, kale, bok choy, almonds
Vitamin D	Needed to absorb calcium; supports muscle strength	Sunlight, oily fish, eggs, fortified foods; many people need a supplement (ask your doctor)
Fiber	Supports gut health, steady blood sugar, fullness and healthy cholesterol	Vegetables, fruit, oats, barley, beans, lentils, whole grains, nuts, seeds
Phytoestrogen-rich foods	May modestly reduce hot flashes for some women	Tofu, tempeh, edamame, soy milk, ground flaxseed, chickpeas, lentils

Your calcium needs rise to about 1,200 milligrams a day from age 51, according to the US National Institutes of Health. Food is the best source, and Chapter 9 shows how to get there with everyday meals. If you think you're falling short, talk to your doctor before taking a supplement.

SOY AND HOT FLASHES

Emerging. Several reviews suggest soy isoflavones may modestly reduce how often hot flashes happen, by around a fifth on average, although results vary between studies. Some women seem to benefit more than others, possibly because of differences in gut bacteria. A recent small trial that combined a plant-based diet with half a cup of cooked soybeans a day reported large reductions in hot flashes, but bigger studies are needed. Soy foods are safe and nutritious for most women, so they're worth including if you enjoy them. Chapter 9 tackles the question of soy and breast cancer.

TRY THIS WEEK

- Add one calcium-rich food to your day, such as a glass of fortified soy milk, a pot of yogurt or a portion of calcium-set tofu.
- Keep a water-based lubricant and a vaginal moisturizer at home, even if you don't need them yet. Discomfort is far easier to deal with early.

Your Menopause Checklist

- ☐ I know the date of my last period, or I've noted it in my tracker.
- ☐ I've talked to my doctor about contraception, if it's relevant to me.
- ☐ I've mentioned any vaginal, bladder or sexual health concerns to my doctor.
- ☐ I eat a calcium-rich food at least twice a day.
- ☐ Every meal is anchored with protein.
- ☐ I wear sunscreen daily.
- ☐ I'm moving most days and doing some strength work twice a week.

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Carmen's milestone was the start of a new chapter, not the end of one. Next: the years beyond, and how to make them your strongest yet.

CHAPTER 4

Post-Menopause: Thriving Beyond

One in two women over fifty will break a bone because of osteoporosis. Most never feel it coming.

> *Linda slipped on the icy step outside her daughter's house in February, put out a hand to catch herself, and heard the crack before she felt it. She was fifty-eight, walked her dog every day, and considered herself "pretty healthy." At the hospital, the doctor looked at the X-ray of her wrist and asked a question nobody had asked her before: had she ever had a bone density scan? She hadn't. Six weeks later, cast off and scan done, she learned the word osteopenia. "I thought bones were just... bones," she told her daughter. "I didn't know they could be fading without a single symptom." Then she asked the question that brought her to this chapter: "Is it too late to do something about it?"*

It isn't. Not for Linda, and not for you.

Once you've passed the twelve-month milestone, you're post-menopausal for the rest of your life. For many women, this stage brings a welcome calm: no more periods, hormones settled, the unpredictable swings of perimenopause behind you. Plenty of women describe their fifties and sixties as a time of new freedom and clarity. At the same time, life with less estrogen brings long-term changes to your bones, heart, muscles and brain that are worth understanding, and these are exactly the areas where food and movement make the biggest difference.

What Fades and What Stays

Every woman is different, but some patterns are common. Mood swings tied to hormonal fluctuation tend to settle, along with breast tenderness and hormone-related migraines, and for most women hot flashes and night

sweats eventually ease, although they can carry on for years. Other symptoms may continue or even appear later: vaginal dryness and discomfort, bladder changes and urinary infections, stiff joints, dry skin and lighter sleep. If anything is still affecting your quality of life, talk to your doctor. There's no age limit on feeling comfortable.

Your Bones Are Alive

Bone is living tissue. All your life, your body breaks down old bone and builds new bone, and estrogen helps keep that process in balance. When estrogen falls, breakdown overtakes building, and bone density starts to slip.

Women can lose up to 20 percent of their bone density in the five to seven years after menopause. Over time, that can lead to **osteoporosis**, a condition in which bones become fragile and break more easily. Hip, spine and wrist fractures are the most common, and hip fractures in particular can seriously affect independence. Osteoporosis is often called a silent disease because there are no symptoms until a bone breaks. That's why prevention matters so much, and why, like Linda, it's never too late to start.

What protects your bones

Bones respond to load. Walking, dancing and climbing stairs all send a "stay strong" signal, but **strength training** sends the loudest one. In trials of supervised, progressive strength training in post-menopausal women with low bone density, bone density and physical function improved. Food matters too: aim for around 1,200 mg of calcium a day, mostly from food, and enough vitamin D to absorb it (many people need a supplement, so ask your doctor). Around half of bone's volume is protein, so protein supports bone strength as well, especially alongside enough calcium.

Smoking and heavy drinking both speed up bone loss. And because the real danger is the fracture, preventing falls counts: balance training, good lighting at home and sensible footwear all reduce your risk. When needed, medical treatment works well. Hormone therapy helps prevent bone loss, and there are several effective medications for osteoporosis. A bone density scan helps your doctor decide what's right for you.

Your Heart Needs You Now

Heart disease is the leading cause of death in women, and the risk rises after menopause. Before menopause, estrogen helps keep blood vessels flexible and cholesterol favorable. Afterward, LDL ("bad") cholesterol tends to climb, blood pressure often creeps up, and the body stores more fat around the abdomen, which is linked to heart disease and type 2 diabetes.

Women's heart attack symptoms can look different from the classic crushing chest pain. They may include breathlessness, nausea, unusual tiredness, discomfort in the jaw, neck, back or arm, or a feeling like indigestion. If you notice these, get urgent medical help.

The good news is that your heart responds to the same habits that run through this whole book. A plant-rich way of eating, built on vegetables, fruit, beans, lentils, whole grains, nuts and seeds, has some of the strongest evidence for heart health, especially in its Mediterranean form. Soluble fiber from oats, barley, beans and lentils helps lower LDL cholesterol. Swapping saturated fats from processed meats, fried foods and pastries for olive oil, nuts, seeds, avocado and oily fish helps too, and cooking more at home cuts the salt that drives blood pressure up. Add around 150 minutes of moderate activity a week, such as brisk walking, and don't smoke, keep alcohol low, manage stress and protect your sleep.

Muscle Is Your Retirement Plan

From around the age of 30 we gradually lose muscle mass and strength, a process that speeds up around menopause and is called **sarcopenia** when it becomes significant. Muscle matters far beyond appearance. It gets you up from a chair, carries your shopping, keeps your balance, lets you play with grandchildren and keeps you independent. It also helps control blood sugar and supports your bones.

Two things protect muscle better than anything else:

1. **Strength training** at least twice a week, using your body weight, bands, free weights or machines. Part Six includes two short no-equipment sessions a week.
2. **Enough protein,** spread through the day. Many experts now suggest that older adults aim for about 1.0 to 1.2 grams per kilogram of body

weight daily, possibly more if you're very active or recovering from illness, with around 25 to 30 grams at each meal. In Anchor, Add, Adapt terms, that's your anchor (Chapter 9).

Balance naturally declines with age too, but it improves quickly with practice. Standing on one leg while you brush your teeth, and the balance poses in the yoga plan, make a real difference.

Keeping Your Brain Sharp

Many women worry about memory during the transition. The brain fog of perimenopause is real, but research suggests it's usually temporary, with memory and concentration often improving after menopause.

Looking further ahead, women are more likely than men to develop dementia, and researchers are actively studying the role of hormones. There's still a lot to learn, but the habits that protect your heart also protect your brain: regular physical activity; a Mediterranean-style diet full of vegetables, berries, leafy greens, beans, nuts, olive oil and fish; good sleep; keeping blood pressure, blood sugar and cholesterol in check; staying socially connected and mentally engaged; protecting your hearing (hearing loss is a recognized risk factor for dementia); not smoking; and keeping alcohol low.

What to Put on Your Plate Now

Nutrient	Why it matters now	Good food sources
Vitamin B12	Absorption declines with age and with some medicines (such as metformin and acid reducers); low levels cause fatigue and nerve problems	Fish, meat, eggs, dairy, fortified plant milks and cereals, nutritional yeast
Calcium	Supports bone strength	Dairy, calcium-set tofu, fortified plant milks, canned fish with bones, leafy greens
Vitamin D	Helps absorb calcium; supports muscle	Oily fish, eggs, fortified foods; often a supplement
Omega-3 fats	Support heart and brain health	Salmon, sardines, mackerel, trout, walnuts, ground flaxseed, chia seeds

Nutrient	Why it matters now	Good food sources
Protein (a little more)	Protects muscle and bone with age	Spread across every meal and snack

Your iron needs fall after menopause, from around 18 mg to 8 mg a day, because you're no longer losing blood every month. Unless your doctor advises otherwise, most post-menopausal women don't need an iron supplement, which is why many multivitamins for women over 50 contain little or none.

Know Your Numbers

Midlife is a great time to get a clear picture of your health, so you and your doctor can spot changes early, when they're easiest to deal with.

Check	Why it matters	How often (general guide)
Blood pressure	Rarely causes symptoms but raises the risk of heart disease and stroke	At least once a year, or as advised
Cholesterol (lipid panel)	LDL cholesterol often rises after menopause	Every few years, or more often if raised
Blood sugar (HbA1c or fasting glucose)	Catches prediabetes and diabetes early	Every few years from midlife, or as advised
Bone density (DXA) scan	Measures bone density and fracture risk	Recommended from 65, or earlier if you have risk factors
Waist measurement	A simple marker of abdominal fat and metabolic risk	Whenever you like, as a check-in
Vitamin D, B12, thyroid	Common deficiencies and conditions in midlife	If you have symptoms or risk factors
Cancer screening	Breast, cervical and colorectal screening save lives	As your national screening program recommends

Earlier bone density testing is often advised if you went through menopause early, have a low body weight, smoke, have a parent who broke a hip, take

long-term steroids, or have broken a bone in a minor fall, as Linda did. Recommendations vary by country, so ask your doctor what's right for you.

TRY THIS WEEK

- Book a check-up if you haven't had your blood pressure and cholesterol checked in the past year.
- Stand on one leg for 30 seconds on each side while you brush your teeth, holding the counter lightly if you need to.

CHAPTER SUMMARY

- Post-menopause is a settled new stage. Some symptoms ease; others, like vaginal dryness, may continue and are very treatable.
- Bone loss speeds up after menopause. Strength training, calcium, vitamin D, protein and fall prevention are your best protection.
- Heart disease risk rises after menopause. Plant-rich eating, fiber, healthy fats, movement and not smoking all help.
- Muscle is one of your greatest assets for healthy aging. Protect it with strength training and protein at every meal.
- Know your numbers: blood pressure, cholesterol, blood sugar and, when appropriate, bone density.

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Bones, heart and muscle are only half the story. The changes many women find hardest to talk about happen in the mind.

CHAPTER 5

Mind and Mood: The Emotional Side of Menopause

What if the anxiety that arrived out of nowhere at forty-six didn't come out of nowhere at all?

> *Aisha was standing in the cereal aisle when the tears came. Nothing had happened. The store had run out of the brand her youngest liked, that was all. But suddenly she was crying behind her sunglasses, her cart half full, unable to stop. She was forty-nine, a nurse who had kept her head through twenty years of emergencies, and lately she didn't recognize herself: short-tempered with her husband, lying awake with a knot of dread she couldn't explain, forgetting words in the middle of sentences. She'd begun to wonder whether she was depressed, or worse, whether something was wrong with her brain. It didn't occur to her, not once, that it might be her hormones.*

Ask women what surprised them most about perimenopause and many won't mention hot flashes. They'll talk about anxiety that came from nowhere, sudden rage over small things, crying in the supermarket, or a flatness that made it hard to enjoy anything. They'll talk about losing words mid-sentence and quietly fearing the worst.

These emotional and cognitive changes are among the most common parts of the menopause transition, and among the most often dismissed. This chapter is here to name them, explain them and point you toward help.

Why Your Mood Changes With Your Hormones

Estrogen and progesterone have powerful effects on the brain. Estrogen influences serotonin, dopamine and other chemical messengers involved in mood, motivation and emotional resilience. Progesterone is naturally calming, partly through a substance it produces called allopregnanolone,

which acts on the same brain receptors as some anti-anxiety medicines. When both hormones swing and fall, your brain has to recalibrate, and for some women that shows up as new or worsening anxiety, sometimes with a racing heart or a constant sense of dread; irritability and a much shorter fuse; low mood, tearfulness or a loss of joy; and a slipping confidence that makes once-manageable things feel overwhelming.

Research shows that the risk of depression rises during perimenopause, even in women who've never been depressed before. Women with a history of depression, postnatal depression or severe premenstrual mood symptoms (PMDD) seem to be especially sensitive to these hormonal shifts.

Hormones are only part of the story, though. Poor sleep, hot flashes, chronic stress and the sheer load of midlife all weigh on mood, and so does how we feel about aging and our changing bodies. Usually it's a mix, and that's actually good news, because it means there are several ways to feel better.

When Big Feelings Feel Bigger

Many women describe a kind of anger in perimenopause they've never felt before: sudden, intense and out of proportion to whatever set it off. Others find themselves in tears several times a day. These moments can be frightening, and they often leave a trail of guilt.

Please be gentle with yourself here. These reactions are often a sign that your nervous system is running on empty, from hormonal shifts, sleep loss and stress all at once. In the moment, a few things can help:

- **Lengthen your out-breath.** Breathe in for four and out for six. A longer exhale switches on your body's calming response within minutes. The breathing practices in Part Six are designed for exactly this.
- **Name it.** Saying, to yourself or someone close, "this is my hormones talking, and I need a minute" can take the heat out of a situation.
- **Step away.** A short walk, a glass of cold water or a few minutes alone can let the wave pass.
- **Eat something.** Low blood sugar makes anger and tears much worse, and a snack with protein can steady things surprisingly fast.
- **Repair afterward.** Everyone loses their temper sometimes. A simple apology and explanation is enough.

If rage or tears are frequent or intense, or they're straining your relationships, talk to your doctor. Both hormonal and non-hormonal treatments can help.

Where Did That Word Go? Brain Fog

Forgetting names, losing your train of thought, walking into a room and forgetting why, struggling to focus at work: brain fog is one of the most common and upsetting parts of the transition, and many women quietly fear early dementia, as Aisha did.

The reassuring part is that research, including findings from the SWAN study, suggests that the changes in memory and processing speed during perimenopause are generally mild and temporary, and tend to improve after menopause. Brain fog is also closely tied to poor sleep, hot flashes, stress and low mood, all of which can be treated.

So protect your sleep wherever you can (Chapter 7), keep your blood sugar steady with regular, anchored meals (Chapter 10), drink enough water, since even mild dehydration affects concentration, and keep moving, because exercise increases blood flow to the brain. Use lists, calendar reminders and a notebook by the bed without a shred of guilt; that's good strategy, not failure. And where you can, do one thing at a time, because multitasking gets harder when you're tired.

If memory problems are getting steadily worse, are affecting your ability to manage daily life, or are being noticed by others, see your doctor to rule out other causes.

Who Am I Now?

Menopause can stir up big questions. Who am I now? What does it mean to no longer be able to have children? Why don't I recognize the woman in the mirror? Our culture often ties women's value to youth and appearance, so it's no wonder many of us feel a sense of loss. And yet many women also describe midlife as a liberation: caring less about what others think, knowing what matters, and finally having the confidence to ask for it.

Self-compassion is one of the most useful tools for this stage of life. It simply means treating yourself with the kindness you'd offer a good friend.

Research links it to lower anxiety and depression, and to healthier eating and exercise habits, because we tend to look after the things we value. I started practicing it, awkwardly, at forty-six, by catching the voice in my head that called me "useless" when I forgot something and asking whether I'd ever say that to my daughter. I wouldn't. Slowly, the voice got quieter.

You can start in the same way: notice your inner voice, and if you wouldn't say it to a friend, try not to say it to yourself. Talk about your body in terms of what it does for you, not only how it looks. Swap "I should" for "I'd like to." Spend time with people who make you feel good about yourself. And give some thought to what you want this next chapter of your life to hold.

Talking About It at Home and at Work

Menopause doesn't just affect you; it touches the people around you. Partners may not understand why you're tired, irritable or less interested in sex. Children notice the shorter fuse. Colleagues may have no idea that a hot flash in a meeting is exhausting, or that you were awake half the night.

You don't have to share everything, but a simple explanation helps others understand and support you:

- *"I'm going through perimenopause. My sleep is really disrupted right now, so I'm more tired than usual."*
- *"When I get a hot flash, I just need a few minutes and some fresh air."*
- *"I'm not angry with you. My hormones are making everything feel more intense right now, and I'm working on it."*

At work, many employers now have menopause policies or support, and practical changes can make a big difference: a desk fan, access to cold water, a later start after a bad night, the option to work from home, or a more breathable uniform. If you feel comfortable, talk to your manager or HR about what would help.

When to Reach Out for Help

Emotional symptoms deserve the same care as physical ones. Effective support is available, including hormone therapy, cognitive behavioral therapy (CBT), antidepressants, counseling and peer support, and your doctor can help you choose. Please seek help promptly if you've felt

persistently low, hopeless or numb for more than two weeks; if you've lost interest or pleasure in most things; if you're struggling to function at work or at home; if you're using alcohol or other substances to cope; if you're having panic attacks; or if you have thoughts of harming yourself or feel others would be better off without you.

> **IF YOU'RE IN CRISIS**
>
> If you're having thoughts of suicide or self-harm, please reach out for help now. Call your local emergency number, go to your nearest emergency department, or contact a crisis line. In the United States, call or text **988** to reach the 988 Suicide & Crisis Lifeline at any time. In the UK and Ireland, call Samaritans on **116 123**. You don't have to face this alone.

> **TRY THIS WEEK**
>
> - Each evening, write down one thing you're grateful for and one thing you did well today, however small.
> - Practice the longer out-breath for two minutes, twice a day: in for four, out for six.

Reflection Prompts

Take a few quiet minutes with a notebook and consider:

1. Which emotional changes have I noticed in the past year?
2. What situations or times of day make them better or worse?
3. Who could I talk to about what I'm experiencing?
4. What's one thing I could take off my plate this month?
5. What do I want this next chapter of my life to feel like?

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Aisha eventually found the words for what she was feeling, and a doctor who listened. Next: the treatments that can help, and how to ask for them.

PART TWO

Treatments, Therapies & Lifestyle

You have more options than any generation of women before you. Knowing them is how you get the help you deserve.

CHAPTER 6

Treatments and Therapies: Knowing Your Options

In 2002, one study's headlines frightened millions of women away from the most effective treatment for hot flashes. What did it actually find?

> *Mei was fifty-three and hadn't slept through the night in eighteen months. Her doctor had mentioned hormone therapy twice, and twice Mei had said no. She remembered the headlines from years ago, and her mother's voice on the phone: "Hormones give you cancer. Don't let them talk you into it." So she tried everything else: black cohosh capsules, a cooling pillow, cutting out coffee, a sleep app with whale sounds. Some nights were a little better. Most weren't. One evening her younger sister, who had started hormone therapy a year earlier, sat her down. "You're allowed to ask questions," she said. "You're not allowed to keep suffering because of a newspaper story from 2002." Mei booked an appointment the next morning, with a list.*

Nutrition and lifestyle can make a real difference to how you feel during the transition, and the rest of this book will show you how. But for many women, food and exercise alone aren't enough, especially for moderate to severe hot flashes, night sweats, mood changes or vaginal symptoms. There's no prize for suffering in silence, and choosing medical treatment isn't a failure.

This chapter gives you a balanced, evidence-informed tour of the main options, so you can talk them through confidently with your healthcare provider. It isn't a substitute for personal medical advice: the right choice depends on your symptoms, your health history, your preferences and your values.

> **HOW TO READ THIS CHAPTER**
>
> I've labeled the evidence as **established** (backed by strong, consistent research and major guidelines) or **emerging** (promising, but not yet settled). Treatments and guidelines change as research moves on, and availability and brand names differ between countries.

Hormone Therapy: The Treatment With the Strongest Track Record

Menopausal hormone therapy, often called hormone replacement therapy (HRT), replaces some of the hormones your ovaries no longer make in the same amounts. It's the **most effective treatment for hot flashes and night sweats** (established), and it also helps prevent bone loss and fractures.

What really happened in 2002

That year, early results from a large American study called the Women's Health Initiative (WHI) were widely reported as showing that hormone therapy raised the risk of breast cancer, heart disease and stroke. Prescriptions collapsed around the world almost overnight.

Since then, much more careful analysis has revealed a more nuanced picture. The women in the WHI were 63 on average, many years past menopause, and many already had risk factors. For healthy women who start hormone therapy before 60, or within ten years of menopause, the benefits generally outweigh the risks. That's now the position of The Menopause Society and other major organizations (established).

The different types, and who they may suit

Estrogen is the ingredient that relieves symptoms. It can be taken as a tablet, or through the skin as a patch, gel or spray, which doctors call *transdermal* estrogen. If you still have a uterus, you also need **a progestogen** (progesterone or a similar hormone) to protect the uterine lining from overgrowth, which could otherwise raise the risk of endometrial cancer; that can be a tablet, part of a combined patch, or a hormonal IUD. Women who've had a hysterectomy can usually take **estrogen alone**. And **testosterone** is

sometimes prescribed for women whose low sexual desire is causing distress, once other factors have been addressed.

Many women notice relief from hot flashes and night sweats within a few weeks, and often better sleep, mood and quality of life. Hormone therapy also prevents bone loss and lowers fracture risk, and for some women it eases vaginal dryness and joint aches.

The risks, in proportion

- **Blood clots and stroke.** Estrogen tablets slightly increase the risk of blood clots. Transdermal estrogen at standard doses doesn't appear to raise this risk in the same way (established), so it's often preferred for women with risk factors.
- **Breast cancer.** Combined therapy (estrogen with a progestogen) is linked to a small increase in breast cancer risk that rises with longer use, while estrogen-only therapy appears to carry little or no increase. For context, the extra risk from combined therapy is similar to or smaller than the extra risk linked to regularly drinking two or more alcoholic drinks a day, or to obesity.
- **Side effects** such as breast tenderness, bloating, headaches or irregular bleeding are common in the first few months. They often settle, or can be eased by changing the dose or type.

Hormone therapy usually isn't recommended for women who've had breast cancer, certain types of blood clots, stroke, heart disease or active liver disease, although specialists may advise differently in individual cases.

HORMONE THERAPY AT A GLANCE

Established: Hormone therapy is the most effective treatment for hot flashes and night sweats, and it prevents bone loss. For most healthy women under 60 or within 10 years of menopause, benefits outweigh risks. Women with premature or early menopause are generally advised to take it until at least the average age of menopause, unless there's a reason not to.

Emerging: Whether starting hormone therapy early has lasting benefits for the heart or brain is still being studied. It isn't currently recommended solely to prevent heart disease or dementia.

"Bioidentical" hormones: read the label

You may see compounded "bioidentical" hormones marketed as more natural or safer. Some regulated, approved products are chemically identical to your body's own hormones (such as estradiol and micronized progesterone), and these are widely prescribed. Custom-compounded products made in individual pharmacies, though, aren't tested and regulated in the same way, so their dose, purity and safety can vary. Major menopause organizations recommend regulated, approved products wherever possible.

Comfort Where It Counts: Vaginal Estrogen

For vaginal dryness, painful sex and bladder symptoms (genitourinary syndrome of menopause, from Chapter 3), **low-dose vaginal estrogen** works very well (established). It comes as a cream, a small tablet or pessary, or a soft ring that stays in place for three months.

Because the dose is very low and acts locally, only tiny amounts reach the bloodstream. That's why it's considered safe for most women, can be used long-term, and doesn't usually need a progestogen alongside it. You can use it together with systemic hormone therapy, and many women who can't or don't want to take systemic hormones can still use it. If you've had breast cancer, discuss it with your oncology team.

Other options include non-hormonal moisturizers and lubricants, vaginal DHEA (prasterone), a tablet called ospemifene, and pelvic floor physical therapy.

When Hormones Aren't Right for You

Not every woman can or wants to take hormone therapy, perhaps because of breast cancer or another hormone-sensitive cancer, a history of blood clots or stroke, or simple preference. Happily, there are now several effective non-hormonal medicines for hot flashes and night sweats (established unless noted).

- **Neurokinin receptor antagonists.** A newer class of medicines, including fezolinetant and elinzanetant, works directly on the overactive temperature-control cells described in Chapter 2. In trials they significantly reduced the number and severity of hot flashes. Because

they're new, longer-term safety data are still being gathered, and your doctor may monitor your liver function.

- **Certain antidepressants (SSRIs and SNRIs).** Low doses of medicines such as paroxetine, escitalopram, citalopram, venlafaxine and desvenlafaxine can reduce hot flashes even in women who aren't depressed, and some help with mood and anxiety too. Some SSRIs interact with tamoxifen, so women taking it need specialist advice.
- **Gabapentin.** An anti-seizure and nerve-pain medicine that can reduce hot flashes, particularly at night. It often causes drowsiness, which can actually help if night sweats wreck your sleep.
- **Oxybutynin.** A bladder medicine that can also reduce hot flashes, although it may not suit older women because of possible effects on memory.
- **Clonidine.** A blood pressure medicine with a modest effect on hot flashes; side effects often limit its use.

Weight-Loss Medications: What to Know If You're Using or Considering Them

A newer group of medicines, known as GLP-1 receptor agonists (and related drugs that act on similar hormones), has become widely prescribed for type 2 diabetes and, for some people, for weight management. They're included here for information only, not as a recommendation. Whether they're right for you is a decision for you and your doctor.

What they are, who they may be prescribed for, and questions to ask

These medicines mimic natural hormones that help regulate appetite and blood sugar, so people tend to feel full sooner and for longer. They're usually prescribed for adults with type 2 diabetes, or for weight management in adults whose weight is affecting their health, according to criteria set by regulators and guidelines. If you're thinking about one, useful questions to ask include: Is it suitable for me, given my health history and other medicines? What are the possible side effects, and what should I do if I get them? How will we monitor my progress, including my muscle and bone health? What happens if I stop?

Side effects, muscle loss and what's still unknown

The most common side effects are digestive: nausea, vomiting, diarrhea and constipation, especially when starting or increasing the dose. Less common but more serious effects, including gallbladder problems and inflammation of the pancreas, have also been reported, and your doctor will explain what to watch for. In clinical trials, a substantial share of the weight lost was lean mass, which includes muscle. That matters for midlife women, who are already losing muscle more quickly, and there's still limited research on these medicines specifically in women going through the menopause transition, including their long-term effects on bone.

Eating well and staying strong if you choose this route

If you and your doctor decide on this path, the principles in this book become even more important. Anchor every meal with protein, around 25 to 30 grams, and eat it first while your appetite is smallest. Keep up strength training at least twice a week to protect muscle and bone. With smaller portions, make every bite count with vegetables, fruit, beans, whole grains, dairy or fortified alternatives, fish and lean protein. Drink plenty of fluid, increase fiber gradually to help with constipation, and ask whether a registered dietitian can support you.

> **PLEASE NOTE**
>
> This section is general information, not medical advice, and it doesn't cover dosing, which is always set by your prescriber. It's based on information from regulators and guideline bodies such as the US Food and Drug Administration, NICE and the National Institutes of Health. Never start, stop or change a prescription medicine without talking to your doctor.

Changing How Hot Flashes Feel: CBT and Hypnosis

Cognitive behavioral therapy (CBT) is a structured, practical talking therapy that helps you notice and change unhelpful patterns of thinking and behavior. Menopause-specific CBT doesn't usually reduce how *often* hot flashes happen, but it significantly reduces how much they *bother* you and interfere with daily life (established). It also helps with sleep, low mood and anxiety, and NICE guidelines now recommend that it be offered for

menopause symptoms. CBT for insomnia (CBT-I) is the first-line treatment for long-term sleep problems in adults, and it works well during the transition. You can access CBT through a therapist, a group program, a book or an evidence-based online course.

Clinical hypnosis, delivered by a trained practitioner or through guided recordings, has also been shown to reduce hot flashes in trials, and The Menopause Society recommends it as a non-hormonal option (established).

The Supplement Aisle, Decoded

The supplement aisle is full of products promising menopause relief. Some are harmless but ineffective, some have limited evidence, and a few can interact with medicines or cause side effects. "Natural" doesn't always mean safe, and supplements aren't tested and regulated as strictly as medicines.

Supplement	What the evidence says
Soy isoflavones	Emerging: may modestly reduce hot flash frequency for some women; soy foods are a good first step
Black cohosh	Mixed and inconsistent results; rare reports of liver problems; not recommended by The Menopause Society
Red clover, dong quai, evening primrose oil, wild yam	Little or no good evidence of benefit; wild yam creams don't convert to progesterone in the body
St. John's wort	Some evidence for mood, but serious interactions with many medicines, including tamoxifen, blood thinners and oral contraceptives
Vitamin D	Established: important for bone health; many people need a supplement, especially in winter
Calcium	Best from food; supplements may be advised if your diet falls short
Omega-3 (fish oil)	Good for heart health as part of the diet; doesn't appear to reduce hot flashes

Creatine, collagen and magnesium: hype or help?

These three are hugely popular with women in midlife. **Creatine** (emerging) is well researched in athletes; it helps muscles produce energy for short bursts of effort, and in older adults, combining it with strength training may lead to bigger strength gains than training alone. Research in menopausal women specifically, including possible effects on bone, mood and the brain, is still limited and mixed. It appears safe for most healthy people at typical doses of around 3 to 5 grams a day, but check with your doctor if you have kidney problems.

Collagen (emerging, limited) may modestly improve skin hydration and elasticity in some studies, but many are funded by manufacturers, and better independent research is needed. It's a protein, but it lacks some essential amino acids, so it can't replace protein from food when it comes to building muscle.

Magnesium (emerging, limited) is essential for muscle and nerve function, and many people don't get enough from food. Evidence that supplements improve sleep or menopausal symptoms is limited, so a food-first approach, with nuts, seeds, beans, whole grains and leafy greens, is a safe place to start. Supplements can cause digestive upset and interact with some medicines.

Before taking any supplement, check with your doctor or pharmacist, especially if you take prescription medicines, have a medical condition or have had cancer.

Feeling Unheard? How to Get the Appointment You Need

Many women say they've been dismissed, misdiagnosed or told "it's just your age." Menopause training for healthcare professionals has historically been thin, though it's improving. I sat through one of those appointments myself, and I walked out with nothing but a leaflet. The second time, with a different doctor, I went in with a two-month symptom diary and a written list of questions, and it changed everything. You deserve to be heard, and a little preparation goes a long way:

1. **Prepare.** Bring your symptom diary or a list of symptoms, how often they happen and how they affect your life. Be specific: "I wake three or

four times a night drenched in sweat, and I've had to cancel work twice this month" is far more powerful than "I'm not sleeping well."

2. **Say the word.** Ask directly: "Could this be perimenopause or menopause?"
3. **Prioritize.** If time is short, start with the one or two symptoms that bother you most.
4. **Ask about every option.** "What are all my treatment options, hormonal and non-hormonal? What are the benefits and risks for someone like me?"
5. **Bring support.** A partner or friend can help you remember what was said.
6. **Take notes,** or ask whether you can record the conversation.
7. **Ask for a follow-up** to review how a treatment is working, typically after three months.
8. **Get a second opinion** if you feel unheard. You can ask to see a clinician with a special interest in menopause; The Menopause Society has a directory of certified practitioners.

Questions to Take to Your Doctor

Photograph or copy this list for your next appointment, as Mei did.

- ☐ Are my symptoms likely to be caused by perimenopause or menopause?
- ☐ Do I need any tests, and why?
- ☐ Is hormone therapy a suitable option for me? What are the benefits and risks in my situation?
- ☐ Which type of hormone therapy would you recommend, and why?
- ☐ What non-hormonal options could help my symptoms?
- ☐ Could vaginal estrogen help with dryness, discomfort or bladder symptoms?
- ☐ Could any of my current medicines or supplements affect my symptoms or interact with treatment?
- ☐ Do I need a bone density scan, or cholesterol, blood sugar or thyroid tests?

- ☐ Can you refer me for CBT, pelvic floor therapy or a menopause specialist?
- ☐ When should we review how things are going?

CHAPTER SUMMARY

- Hormone therapy is the most effective treatment for hot flashes and night sweats and protects against bone loss. For most healthy women under 60 or within 10 years of menopause, the benefits outweigh the risks.
- Transdermal estrogen carries a lower risk of blood clots than tablets, and women with a uterus also need a progestogen.
- Low-dose vaginal estrogen is safe and effective for vaginal and bladder symptoms for most women.
- Effective non-hormonal medicines include neurokinin receptor antagonists, certain antidepressants and gabapentin. CBT and clinical hypnosis also help.
- If you use a weight-loss medication, protect your muscle with protein at every meal and regular strength training.
- Most herbal supplements have little evidence behind them, and some carry risks. Always check with your doctor first.

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Treatment can turn the volume down on symptoms. The habits in the next chapter help keep it down, and they protect you for decades.

CHAPTER 7

Everyday Habits That Make a Difference

After a meal, your muscles absorb most of the sugar in your blood. Every time you strengthen them, you're improving the system that does it.

> *Jo started with two cans of tomatoes. She was fifty-five, had never set foot in a gym, and found the idea of one faintly terrifying. But her doctor had used the word "osteopenia," and a friend had shown her a ten-minute routine she did in her kitchen. So on a Tuesday evening Jo stood between the counter and the fridge, a can in each hand, and did her first set of squats while the pasta water boiled. It felt ridiculous. Her thighs complained the next day. She did it again on Friday. Three months later she'd graduated to a pair of dumbbells from a thrift store, could carry all the groceries in from the car in one trip, and had stopped dreading the stairs at work. "I don't love exercise," she says. "But I love what it's given back."*

Alongside nutrition, a handful of everyday habits have strong evidence for helping women feel better through the transition and for protecting long-term health. None of them requires perfection, expensive equipment or hours of free time. They work best when they become part of ordinary life, the way Jo's squats fit between the counter and the fridge.

Strength Training: The Habit That Pays You Back

If you take one habit from this chapter, make it this one. Strength training (also called resistance training) means working your muscles against resistance: your own body weight, resistance bands, dumbbells, cans of tomatoes or gym machines. It's one of the most effective things you can do in midlife (established), because it builds and maintains the muscle that

naturally declines with age, helps maintain bone density, improves blood-sugar control and insulin sensitivity, supports a healthy body composition, improves balance and lowers your risk of falls, and lifts mood, sleep and confidence.

How much is enough?

Guidelines from the World Health Organization recommend muscle-strengthening activities that work all the major muscle groups on **two or more days a week**. That's it. Two sessions of 15 to 30 minutes can make a real difference.

Starting from zero

Start where you are. Bodyweight exercises such as standing up from a chair, wall push-ups, glute bridges and step-ups are an excellent beginning, and Part Six includes two 15-minute no-equipment sessions a week. Move slowly and with control, because good form matters more than the number of repetitions. When an exercise starts to feel easy, make it a little harder: add a few repetitions, slow the movement down, or add a light weight or band. By the last two or three repetitions of a set it should feel hard, while you can still keep good form. And leave a day between sessions for the same muscles, because muscles grow stronger while they recover.

> **SAFETY FIRST**
>
> If you have osteoporosis, a previous fracture, joint problems, high blood pressure or another health condition, check with your doctor or a physical therapist before starting a new program. Many women with osteoporosis can and should strength train, with the right guidance.

Moving for Your Heart

Aerobic exercise, or cardio, is anything that raises your heart rate and makes you breathe a little harder: brisk walking, cycling, swimming, dancing, hiking or a class. It protects your heart, lowers blood pressure, improves cholesterol, supports a healthy weight and lifts your mood.

Guidelines recommend at least **150 minutes of moderate activity a week** (for example, 30 minutes on five days), or 75 minutes of vigorous activity, or

a mix. Moderate means you can talk but not sing; vigorous means you can only manage a few words before pausing for breath. It doesn't have to happen all at once. Ten-minute walks after meals count, and walking after you eat has a bonus: it helps flatten blood-sugar spikes.

Beyond formal exercise, all the movement in your day adds up, and long stretches of sitting are linked to poorer health even in people who exercise. Stand up and move for a few minutes every hour, take phone calls on your feet, use the stairs, park a little further away, garden, play with the dog, dance in the kitchen. It all counts.

Sleep: Cooler, Darker, Calmer

Sleep problems affect a large share of women during the transition, and poor sleep makes almost every other symptom feel worse. Hot flashes and night sweats may need medical treatment, but good sleep habits help everyone.

Keep it cool. Aim for a bedroom around 60 to 67°F (15 to 19°C). Choose breathable cotton, linen or bamboo bedding and nightwear, or moisture-wicking fabrics, and layer your bedding so you can throw a layer off or pull one on in seconds. Keep a glass of cold water and a small fan by the bed, and try a cool shower before turning in.

Set your body clock. Go to bed and get up at roughly the same time every day, weekends included. Get outside in natural daylight in the morning, even for ten minutes, which helps set your internal clock. Keep naps short, under 30 minutes, and before mid-afternoon.

Wind down on purpose. Give the last 30 to 60 minutes before bed to something calming: dim the lights, read, stretch, take a warm (not hot) bath or try one of the evening practices in Part Six. Put screens away an hour before bed, or at least switch them to night mode. A notebook on the nightstand gives worries and to-do lists somewhere to go.

Mind your evening plate and glass. Avoid caffeine after early afternoon, limit alcohol (it disrupts sleep and can trigger night sweats), and try not to eat a large or very spicy meal in the two to three hours before bed. If you wake up hungry, a small snack with protein and slow carbohydrates, like a little Greek yogurt with oats or whole-grain toast with nut butter, may help.

If you've been lying awake for around 20 minutes, get up, go to another dimly lit room and do something calm until you feel sleepy, then go back to bed. This helps your brain relearn that bed is for sleeping. If insomnia lasts for weeks, ask your doctor about CBT for insomnia (Chapter 6), the most effective long-term treatment.

> **IS IT SLEEP APNEA?**
>
> Sleep apnea, in which breathing repeatedly stops and starts during sleep, becomes much more common after menopause. Signs include loud snoring, gasping or choking at night, waking with a headache or dry mouth, and feeling exhausted despite a full night in bed. It's very treatable, so talk to your doctor if this sounds familiar.

Stress and the Art of Doing Nothing

Midlife is often one of the busiest stretches of life. Many of us are juggling work, children, aging parents, relationships and our own changing health all at once. Chronic stress keeps cortisol elevated, which can worsen sleep, increase cravings and belly fat, and make hot flashes and mood symptoms harder to cope with.

You can't remove stress from your life, but you can build in moments of recovery that let your nervous system reset. Slow breathing with a longer out-breath switches on the body's "rest and digest" response within minutes. Five to ten minutes of mindfulness or meditation a day can reduce stress and improve mood and sleep, and the 30-day plan in Part Six includes guided practices. Yoga combines movement, breath and relaxation, and studies show it can improve psychological symptoms and quality of life in menopausal women. Time in nature lowers stress. Saying no protects your time and energy. And rest without guilt is part of the plan: it's how your body and mind repair.

Rethinking What's in Your Glass

Alcohol

Many women find their relationship with alcohol changes in midlife. The glass of wine that once felt relaxing may now trigger hot flashes, wreck your

sleep or leave you anxious the next day. Alcohol also raises the risk of breast cancer, even at low levels (established), contributes to bone loss and falls, adds calories without nutrients while stimulating appetite, and can worsen mood and anxiety.

No amount of alcohol is completely risk-free, but if you choose to drink, keeping it low makes a real difference. Current US Dietary Guidelines advise that women who drink have no more than one drink a day, and many health organizations now encourage less. Try several alcohol-free days a week, alternate alcoholic drinks with water, choose smaller glasses, or explore the growing world of alcohol-free drinks. The Tart Cherry Evening Spritz and the other "soothing sips" in Part Four make lovely stand-ins.

Caffeine

Coffee and tea can be part of a healthy diet, and both contain beneficial plant compounds. But caffeine can trigger hot flashes in some women, heighten anxiety and palpitations, and interfere with sleep, since it can linger in your system for many hours. Most healthy adults can safely have up to about 400 mg a day (roughly three to four cups of brewed coffee), but many women in the transition feel better on less. Try having your last caffeinated drink before about 2 p.m., notice whether caffeine affects your hot flashes, sleep or anxiety, and swap some cups for decaf, herbal tea or a Golden Turmeric Latte. Remember that tea, cola, energy drinks and chocolate contain caffeine too.

If you smoke

Smoking is linked to earlier menopause, more severe hot flashes, faster bone loss, heart disease, several cancers and faster skin aging. Stopping at any age brings real benefits, and your heart disease risk starts to fall soon after you quit. You're far more likely to succeed with support, so ask your doctor or pharmacist about stop-smoking medicines, nicotine replacement and free quit services.

The Muscles Nobody Talks About: Your Pelvic Floor

The pelvic floor is a hammock of muscles that supports your bladder, bowel and uterus. Pregnancy, childbirth, aging and lower estrogen can all weaken it. Signs include leaking when you cough, sneeze, laugh or exercise; needing to rush to the bathroom; or a feeling of heaviness in the vagina. These

symptoms are common, but you don't have to live with them. Pelvic floor exercises, often called Kegels, work for many women (established):

1. Sit or lie comfortably. Imagine trying to stop yourself passing wind and stop the flow of urine at the same time. Squeeze and lift those muscles inward and upward.
2. **Slow squeezes:** hold for up to 10 seconds, then fully relax for 10 seconds. Repeat up to 10 times.
3. **Quick squeezes:** squeeze and lift quickly, then relax straight away. Repeat up to 10 times.
4. Do a set of both three times a day. Keep breathing normally and avoid clenching your buttocks or thighs.

It can take three months or more to notice a difference. If symptoms persist, ask for a referral to a pelvic floor physical therapist, who can check your technique and offer other treatments. Vaginal estrogen can also help with bladder symptoms.

People, Purpose and Belonging

Strong social connections are among the most powerful predictors of long-term health and happiness, while loneliness is linked with poorer mental and physical health. Yet midlife can be isolating: children leave home, careers change, relationships shift, and symptoms can make us pull back.

Connection doesn't require a packed social calendar. It might be a regular walk or coffee with a friend, a menopause support group in person or online, a class, a choir, a book club or volunteering, an honest conversation with your partner or family, or a call to an old friend you've lost touch with. Purpose matters too. Many women find midlife is the moment to rediscover an old passion or take on a new challenge. What would you love to learn, make or give in the years ahead?

TRY THIS WEEK

- Put two 15-minute strength sessions in your calendar this week, just as you would an appointment.
- Take a 10-minute walk after your biggest meal of the day on at least three days.
- Set a 2 p.m. caffeine curfew and notice what happens to your sleep.

Your Everyday Habits Checklist

- ☐ I have strength training planned on two days this week.
- ☐ I'm moving for at least 150 minutes a week, in whatever way I enjoy.
- ☐ My bedroom is cool, dark and quiet.
- ☐ I go to bed and wake up at roughly the same time each day.
- ☐ My last caffeine is before early afternoon.
- ☐ I have several alcohol-free days each week.
- ☐ I'm practicing pelvic floor exercises.
- ☐ I have at least one moment of real rest each day.
- ☐ I've connected with a friend or loved one this week.

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Now that you know your options and your habits, it's time for food. But first, a quick check of where you are.

PART THREE

Nourishing Your Changing Body

Food won't fix everything, but it can help with almost everything. This is where your plate starts working for you.

CHAPTER 8

Not Sure of Your Stage? Start Here

You don't need a blood test to know where you are in the transition. Three questions will usually tell you.

> *Sofia had skipped straight to the food chapters. She was forty-five, busy, and had bought the book for the recipes, she told herself, not for "all the hormone stuff." Then she hit the word perimenopause on the first page of nutrition advice and stopped. Was she in it? Her periods were still coming, more or less. She'd had a few hot nights last summer, but it had been a hot summer. She was tired, but who wasn't? Standing at the kitchen counter with a cooling cup of coffee, she flipped back a few pages, looking for something simple. Just tell me where I am, she thought, and what to eat.*

This chapter is for Sofia, and for you if you've jumped straight here. It'll help you work out your stage in a couple of minutes and point you to the parts of the book that will help you most.

Three Questions

1. **Have you gone 12 months or more without a period** (with no other explanation, such as pregnancy, a hormonal IUD or certain medicines)?
   - Yes: you're **post-menopausal**.
   - No: go to question 2.
2. **Have your periods changed** in timing, flow or frequency, or have you started noticing hot flashes, night sweats, broken sleep or new anxiety?
   - Yes: you're probably in **perimenopause**.
   - No: you may not have started the transition yet, and the habits in this book will give you a great head start.
3. **Have you had a hysterectomy or your ovaries removed, or do you use a hormonal IUD?** Then periods can't guide you. Use your

symptoms and age instead, and talk to your doctor. If both ovaries were removed, you're in surgical menopause.

In this book, the year or so either side of your final period is often called the "menopause" stage, because that's when many symptoms peak. Don't worry about labeling yourself precisely. The stages blend into one another, and the core of eating well stays the same throughout. Only the emphasis shifts.

Your Stage on One Page

	Perimenopause	Around menopause	Post-menopause
Main focus	Steady energy, mood, sleep and cycle changes	Hot flashes, sleep, joints, body composition	Long-term bone, heart, muscle and brain health
Priority nutrients	Iron (if periods are heavy), B vitamins, magnesium, protein, fiber	Protein, calcium, vitamin D, fiber, phytoestrogens	Protein, calcium, vitamin D, vitamin B12, omega-3s
Symptoms to target	Energy dips, cravings, anxiety, PMS, bloating	Hot flashes, night sweats, sleep, joint aches	Lingering hot flashes, joint stiffness, dry skin
Foods to lean on	Lentils, beans, leafy greens, lean meat or tofu, whole grains, nuts and seeds	Soy foods, flaxseed, dairy or fortified alternatives, oily fish, colorful vegetables	Oily fish, dairy or fortified alternatives, eggs, legumes, berries, nuts, olive oil
Iron needs	Around 18 mg a day	Around 18 mg until periods stop	Around 8 mg a day
Calcium needs	Around 1,000 mg a day	Around 1,200 mg a day from age 51	Around 1,200 mg a day

When to Eat and Drink

The rhythm that suits most women at every stage is simple: **three anchored meals a day, plus a snack if you need one.** Regular meals keep blood sugar steady, calm cravings and support better energy and mood.

In **perimenopause,** don't skip breakfast if you tend to crash mid-morning, and if premenstrual cravings hit, plan a satisfying late-afternoon snack rather than relying on willpower. **Around menopause,** keep evening meals

moderate and finish eating two to three hours before bed if night sweats are a problem, and notice whether hot drinks, spicy food or alcohol in the evening set off hot flashes. In **post-menopause,** make sure every meal carries a good portion of protein, around 25 to 30 grams, to protect muscle; appetite can shrink with age, so make each bite count.

Hydration matters at every stage, and even more if hot flashes and night sweats leave you damp. The US National Academies suggest women need about 2.7 liters (around 11 cups) of total water a day from all drinks and foods, with roughly a fifth usually coming from food. In practice, aim for six to eight glasses of fluid a day, more in hot weather or when you exercise. Water, milk, herbal tea, and even tea and coffee all count. Pale yellow urine is a good sign you're drinking enough.

Where to Go Next

If you want to...	Turn to...
Learn the method behind every meal in this book	Chapter 9: Anchor, Add, Adapt
Find food strategies for a specific symptom	Chapter 10: Eating for Your Symptoms
Understand weight and body changes, kindly	Chapter 11: Rethinking Weight in Midlife
Make healthy eating work on a busy schedule	Chapter 12: The Busy Woman's Kitchen
Start cooking right away	Part Four: The Recipes
Follow a ready-made plan	Part Five: The 12-Week Meal Plan

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Sofia found her stage in three questions. Now for the three words that will shape everything she, and you, put on the plate.

CHAPTER 9

Anchor, Add, Adapt: Your Everyday Framework

How much protein was in your breakfast this morning? For most women, it's less than half of what midlife muscles need.

> *Keisha's breakfast hadn't changed in fifteen years: a slice of toast with butter, eaten standing up, and a large coffee in the car. It had always worked. Lately it didn't. By 10:30 she was shaky, short-tempered and raiding the vending machine at the hospital where she managed the front desk, and by evening she was too tired to cook. She was forty-eight. When a friend suggested she "eat more protein," Keisha laughed. "I'm not a bodybuilder." But she tried it for a week: Greek yogurt with berries and walnuts, or two eggs and a slice of whole-grain toast, eaten sitting down. On Thursday, at 10:30, she realized she hadn't thought about the vending machine once.*

There's no single "menopause diet," and you won't find a list of forbidden foods here. Instead you'll find one method that works for any cuisine, budget or dietary preference, built around the nutrients your body needs most right now. It's three words long: **Anchor, Add, Adapt.**

If Food Has Felt Complicated

Many of us arrive at midlife carrying decades of diet messages: low-fat, low-carb, points, shakes, cleanses, "clean eating." For some, food has become tangled up with guilt, anxiety or a sense of failure. If that's you, please know this chapter isn't another set of rules to get right or wrong.

Anchor, Add, Adapt is a guide, not a test. Some meals will fit it beautifully; others won't, and that's completely fine. A slice of birthday cake, takeout pizza with friends or a bowl of your grandmother's pasta are part of a full and happy life. What matters is the overall pattern of your eating over weeks

and months, not any single meal. If you have a history of disordered eating, or thinking about food causes you real distress, a doctor or registered dietitian can offer personalized support.

The Method in Three Words

Anchor every meal with protein: around 25 to 30 grams, roughly a palm-and-a-half of fish, chicken, tofu or eggs, or a generous bowl of Greek yogurt or lentils.

Add plants: vegetables, fruit, beans, whole grains, nuts and seeds, filling about half your plate and aiming for 30 different plants across the week.

Adapt to where you are: your stage of the transition, your symptoms this week, your appetite and your life.

You can picture it on a plate. Fill half with vegetables (and some fruit), a quarter with your protein anchor, and a quarter with fiber-rich carbohydrates such as whole grains, beans or starchy vegetables. Finish with a little healthy fat, like a drizzle of olive oil, a sprinkle of seeds or a few slices of avocado. The plate is the picture; Anchor, Add, Adapt is the way of thinking that makes it work at breakfast, at your desk and on a Tuesday night when the fridge looks empty.

Plate section	Why it matters in menopause	Examples
Plants (half the plate)	Fiber, antioxidants, potassium and folate for your gut, heart and weight	Spinach, kale, broccoli, peppers, tomatoes, berries, carrots, bok choy
Protein anchor (a quarter)	Protects muscle and bone, steadies blood sugar, keeps you full	Salmon, chicken, eggs, tofu, lentils, Greek yogurt, edamame
Fiber-rich carbs (a quarter)	Steady energy, fiber, B vitamins	Oats, quinoa, brown rice, sweet potato, whole-grain bread, beans
Healthy fats (a little)	Heart and brain health; help you absorb vitamins A, D, E and K	Olive oil, avocado, walnuts, almonds, flaxseed, chia seeds

Anchor: Why Protein Comes First

Think about the breakfast you grabbed this morning. Toast and coffee? That's barely 10 grams of protein, at exactly the stage of life when your muscles need it most. If you change one thing on your plate, make it this.

Protein provides the building blocks for muscle, bone, skin, hair, enzymes and hormones, and it's the most filling of all the nutrients, keeping you satisfied for longer and your blood sugar steady. As estrogen falls, it gets harder for your body to hold on to muscle. Enough protein, especially alongside strength training, is one of the most effective ways to protect your muscle, your metabolism and your independence for decades to come.

I didn't believe this until I tracked my own breakfasts for a week. My "healthy" bowl of granola with a splash of milk came to about 8 grams. No wonder I was starving by eleven.

How much, and when

The official minimum for adults is 0.8 grams of protein per kilogram of body weight per day. Many experts in healthy aging now suggest that adults over 50 may do better on more, around **1.0 to 1.2 grams per kilogram per day**, and possibly higher if you're very active. For a woman weighing 150 pounds (68 kg), that's roughly 68 to 82 grams a day. (If you have kidney disease, check with your doctor before increasing protein.)

Timing matters as much as the total. Most of us eat a little protein at breakfast, a bit more at lunch and most of it at dinner. Muscles seem to respond best when it's spread more evenly, with around **25 to 30 grams at each meal**. That's what anchoring means.

Food	Approximate protein
3.5 oz (100 g) cooked chicken breast	31 g
3.5 oz (100 g) cooked salmon	25 g
3/4 cup (170 g) plain Greek yogurt	15 to 17 g
1/2 cup (113 g) cottage cheese	12 to 14 g
2 large eggs	12 g
3.5 oz (100 g) firm tofu	12 to 17 g

Food	Approximate protein
1 cup (155 g) shelled edamame	17 to 18 g
1/2 cup (100 g) cooked lentils	9 g
1/2 cup (85 g) cooked chickpeas or black beans	7 g
1 cup (240 ml) soy milk	7 to 8 g
1 oz (28 g) almonds or pumpkin seeds	6 to 9 g
2 tbsp peanut butter	7 g

Values vary by brand and preparation; check the label.

Plant proteins like tofu, tempeh, edamame, beans, lentils, nuts and seeds are excellent anchors that bring fiber and other nutrients with them. If you eat mostly or only plants, mix your sources through the day, and lean on soy foods in particular, which contain all the essential amino acids.

> **ANCHOR YOUR BREAKFAST**
>
> - Choose one protein-anchored breakfast this week: Spinach & Feta Egg Muffins, the Greek Yogurt Power Bowl, the Turmeric Tofu & Veggie Scramble, or Golden Flax & Berry Overnight Oats made with Greek yogurt and soy milk.
> - Notice how your energy and appetite feel at 10:30, as Keisha did.

Add: Plants, Fiber and Your Gut

Once your meal is anchored, add plants. Fiber is the part of plant foods your body can't fully digest, and that's exactly why it's so valuable. It travels to your large intestine and feeds the trillions of beneficial bacteria that make up your **gut microbiome**. Along the way it keeps you regular, slows the absorption of carbohydrates so blood sugar stays steadier, helps lower cholesterol (especially the soluble fiber in oats, barley, beans and lentils), and keeps you full.

Most adults eat far less fiber than recommended. Aim for **25 grams or more a day**, which is easiest when every meal includes vegetables, fruit, whole

grains, beans, nuts or seeds. If you're not used to much fiber, increase it gradually over a few weeks and drink plenty of water to avoid bloating.

Thirty plants a week

A target that's far more fun than counting grams: **30 different plant foods a week.** Researchers from the American Gut Project found that people who ate at least 30 different plants a week had more diverse gut bacteria than those who ate ten or fewer. Every vegetable, fruit, whole grain, bean, lentil, nut, seed, herb and spice counts, so a stir-fry with five vegetables, garlic, ginger and sesame seeds gets you eight in one go.

Meet the estrobolome

This next part is fascinating. Some of the bacteria in your gut make enzymes that help decide how much estrogen is recycled back into your bloodstream and how much leaves the body. Scientists call this collection of bacteria the **estrobolome** (emerging). A healthy, diverse microbiome seems to support healthy estrogen balance, and an imbalanced one may disrupt it.

The research is young, and nobody can yet tell you exactly how to "optimize" your estrobolome. But we do know the microbiome thrives on a varied, fiber-rich diet with plenty of plants, and that fermented foods such as yogurt, kefir, kimchi, sauerkraut and miso can increase microbial diversity. Gut bacteria also turn soy isoflavones into a compound called equol, which may explain why some women benefit more from soy than others.

The Fats That Help

Fat is essential. It helps you absorb vitamins A, D, E and K, supports brain and hormone health, and makes food satisfying and delicious. The key is choosing mostly unsaturated fats: monounsaturated fats from extra virgin olive oil, avocado, almonds, cashews and peanuts, and omega-3 fats from oily fish (salmon, sardines, mackerel, trout, herring, anchovies), walnuts, ground flaxseed, chia and hemp seeds, and soy foods.

Omega-3s support heart health and are an important part of the brain. If you eat fish, aim for **oily fish twice a week.** If you don't, include plant sources every day. The body converts only part of the plant form (ALA) into the forms found in fish (EPA and DHA), so vegetarians and vegans may want to ask a doctor or dietitian about an algae-based omega-3 supplement.

It's wise to go easy on **saturated fat,** found mainly in fatty and processed meats, butter, cream, cheese, coconut oil, pastries and fried foods, because it can raise LDL cholesterol, which tends to climb after menopause. You don't need to avoid it completely; just make unsaturated fats your everyday choice.

Soy, Flax and the Phytoestrogen Question

Phytoestrogens are natural plant compounds with a structure similar to estrogen, and they can bind weakly to estrogen receptors in the body. The two main types are **isoflavones,** found mainly in soy foods (tofu, tempeh, edamame, soy milk, miso and soybeans) and in smaller amounts in chickpeas and other legumes, and **lignans,** found in ground flaxseed, sesame seeds, whole grains and some fruits and vegetables.

They're much weaker than your own estrogen and don't act like hormone therapy. Research on hot flashes is promising but mixed (emerging): some women notice a modest improvement after several weeks of eating soy foods regularly, and others notice nothing. Studies typically use two to three servings a day, such as a cup of soy milk, half a cup of edamame and a portion of tofu.

Soy and breast cancer: clearing up the confusion

Many women have been told to avoid soy because of fears about breast cancer. The research now tells a clear story (established). In large population studies, eating soy foods is **not linked to a higher risk of breast cancer**, and in Asian populations with high lifelong soy intakes it's associated with a lower risk. For women who've had breast cancer, eating soy foods doesn't appear to increase the risk of recurrence and may be linked to a lower one. The American Institute for Cancer Research and the American Cancer Society both consider moderate amounts of soy foods safe for women, including breast cancer survivors.

That research is about whole soy foods. Concentrated isoflavone supplements are a different matter, and women with a history of hormone-sensitive cancer should talk to their oncology team before taking them.

The Small Nutrients With Big Jobs

Nutrient	Daily need (women)	Why it matters	Top food sources
Calcium	1,000 mg (to age 50); 1,200 mg (51+)	Bone strength, muscle and nerve function	Milk, yogurt, cheese, calcium-set tofu, fortified plant milks, canned sardines or salmon with bones, kale, bok choy, almonds
Vitamin D	600 IU (to age 70); 800 IU (71+)	Calcium absorption, bone and muscle health	Sunlight on skin, oily fish, egg yolks, fortified milks and cereals; supplements often advised
Magnesium	Around 320 mg	Muscle relaxation, sleep, blood sugar, bone	Pumpkin seeds, almonds, cashews, spinach, black beans, edamame, whole grains, dark chocolate
Iron	18 mg (while periods continue); 8 mg (after menopause)	Energy, oxygen transport, concentration	Lean red meat, poultry, fish, lentils, beans, tofu, fortified cereals, spinach (with vitamin C)
Vitamin B12	2.4 mcg	Energy, nerves, red blood cells	Fish, meat, eggs, dairy, fortified plant milks, nutritional yeast
Folate	400 mcg	Cell repair, mood, heart health	Leafy greens, beans, lentils, asparagus, avocado, fortified grains
Potassium	Around 2,600 mg	Healthy blood pressure	Beans, lentils, potatoes, sweet potatoes, bananas, spinach, yogurt

Daily needs are based on US National Institutes of Health recommendations for adult women and may differ slightly by country.

If your meals follow Anchor, Add, Adapt, you'll meet most of these needs from food. Vitamin D is the main exception for many people, especially in fall and winter or if you're rarely outdoors, and women following a vegan diet need a vitamin B12 supplement. Ask your doctor whether a blood test or supplement is right for you.

Adapt: Make It Yours

The third word is what makes the method last. Adapting means the framework bends to your life instead of the other way around.

Adapt to your stage. In perimenopause, lean on iron-rich foods if your periods are heavy. Around menopause, add soy and flax and keep calcium high. After menopause, nudge your protein up and keep up the oily fish (Chapter 8 has the details).

Adapt to your symptoms. Hot flashes, poor sleep, cravings, bloating and joint pain each respond to specific tweaks, which Chapter 10 walks through one by one.

Adapt to your appetite and your day. Very active, or trying to build muscle? You may need more fiber-rich carbohydrates and protein. Smaller appetite? Make every bite count. Eating out, traveling or cooking for a family? Look for the anchor and the plants, and let the rest go.

Adapt to your culture and your kitchen. Dal and rice, tacos with beans and slaw, congee with egg and greens, a Mediterranean grain bowl: they all fit. There's no single way to eat for menopause.

An anti-inflammatory way of eating

Low-grade, long-term inflammation is linked with heart disease, type 2 diabetes, joint pain and other conditions that become more common after menopause. An anti-inflammatory way of eating isn't a special diet; it looks a lot like the Mediterranean diet, one of the best-researched eating patterns in the world. Eat more vegetables, fruit (especially berries), beans and lentils, whole grains, nuts and seeds, extra virgin olive oil, oily fish, herbs and spices like turmeric, ginger, garlic and cinnamon, and green tea. Eat less ultra-processed food, processed meat, refined grains, sugary drinks and sweets, fried food and alcohol. Every time you add a vegetable, swap a sugary drink for water or reach for olive oil, you're moving your overall pattern in the right direction.

Portions Without a Scale

Weighing and measuring food can feel tedious and, for some of us, stressful. Your hand is a quick, portable guide that's naturally scaled to your body.

Food group	Your hand	Roughly equals	Per meal
Protein anchor	1 to 1.5 palms (thickness and size of your palm)	3 to 5 oz (85 to 140 g) meat, fish or tofu	1 to 1.5 portions
Plants	2 fists	About 2 cups	2 or more
Fiber-rich carbs	1 cupped hand	About 1/2 to 2/3 cup cooked grains or starchy vegetables	1 to 2, depending on hunger and activity
Healthy fats	1 thumb	About 1 tbsp oil, nut butter, nuts or seeds	1 to 2

These are starting points, not limits. Pay attention to your hunger and fullness, and aim to finish meals comfortably satisfied, not stuffed.

Foods to Lean Into

Food	Why it helps	Easy ways to enjoy it
Oily fish	Omega-3s, protein, vitamin D	Sheet-Pan Salmon, sardines on rye toast, smoked salmon bites
Soy foods	Protein, calcium (if calcium-set), isoflavones	Tofu scramble, edamame snacks, soy milk in oats
Beans and lentils	Protein, fiber, folate, magnesium	Dal, chili, soups, salads, hummus
Leafy greens	Calcium, folate, magnesium, vitamin K	Stir into soups, stews, eggs and smoothies
Berries	Fiber, antioxidants	Overnight oats, yogurt bowls, frozen as a snack
Greek yogurt and kefir	Protein, calcium, live cultures	Breakfast bowls, dips, smoothies, sauces
Ground flaxseed	Lignans, omega-3s, fiber	On oats, yogurt and salads; in baking
Nuts and seeds	Healthy fats, magnesium, protein	A small handful as a snack; toasted on salads
Whole grains	Fiber, B vitamins, steady energy	Oats, quinoa, brown rice, barley, rye bread

Food	Why it helps	Easy ways to enjoy it
Extra virgin olive oil	Heart-healthy fats, antioxidants	Dressings, roasting, drizzling
Herbs and spices	Flavor without salt; anti-inflammatory compounds	Turmeric, ginger, garlic, cinnamon, cumin

CHAPTER SUMMARY

- **Anchor** every meal with 25 to 30 grams of protein. It's the single most useful change most women can make.
- **Add** plants to fill half your plate, aiming for 25+ grams of fiber a day and 30 different plants a week.
- **Adapt** the method to your stage, your symptoms, your appetite and your culture.
- Choose mostly unsaturated fats, and eat oily fish twice a week or plant omega-3s daily.
- Soy foods are safe for most women, including breast cancer survivors, and may modestly help hot flashes.
- Your hand is a portable portion guide. Listen to your hunger and fullness.

• • •

Anchor, Add, Adapt is the everyday foundation. Next, we'll put the "adapt" to work on the symptoms that are bothering you most.

CHAPTER 10

Eating for Your Symptoms

The glass of wine that used to help you unwind may be the very thing waking you at 3 a.m.

> *For years, Hannah's evening ritual had been the same: kids in bed, a glass of red, an hour of television. At fifty, she started waking at three every morning, hot and wide awake, heart thudding. She blamed work stress, then the mattress, then the dog. It was her sister-in-law, over dinner one night, who asked casually, "Does it happen on the nights you don't drink?" Hannah didn't know. So she ran an experiment: two weeks, wine only on weekends, with a note on her phone every time she woke up. The pattern was so clear it made her laugh. Weekday nights, she mostly slept through. Fridays and Saturdays, 3 a.m., like an alarm clock.*

Hannah's experiment is the heart of this chapter. Food can't make every symptom disappear, but targeted changes can take the edge off many of them, and the best way to find out what works for you is to test one change at a time.

Each section below focuses on one common symptom and explains how food and daily habits may help, what may make things worse, and one or two things to try. You'll also find recipes from Part Four that suit each symptom. Three things to keep in mind as you read: try one change for two to four weeks and track it in your symptom diary (Appendix A); remember that triggers are personal, so what sets off one woman's hot flashes may do nothing to yours; and treat food as a partner to other support, not a replacement for it. If a symptom is seriously affecting your life, talk to your doctor about all your options (Chapter 6).

Hot Flashes and Night Sweats

Hot flashes start in the brain's temperature control center as estrogen fluctuates and falls (Chapter 2). Diet can't switch them off, but it can help you avoid the things that set them off and support your body overall.

What tends to light the fuse

The most common triggers are alcohol (red wine especially, and particularly in the evening), caffeine in coffee, tea, cola, energy drinks and chocolate, spicy food, hot drinks and very hot food, large meals late in the evening, and sugary foods or refined carbohydrates that send blood sugar swinging.

What can help cool things down

- **Eat regular, anchored meals** with protein, fiber and healthy fats to keep blood sugar steady.
- **Keep evening meals moderate,** and finish eating two to three hours before bed if night sweats disturb your sleep.
- **Stay well hydrated,** especially after a hot flash or night sweat, and keep a bottle of cold water with you.
- **Reach for cooling foods and drinks** when you're warm: cucumber, watermelon, berries, salads, chilled soups, yogurt and iced herbal teas.
- **Include soy foods and ground flaxseed regularly.** Some women find that two to three servings of soy foods a day modestly reduce hot flashes over several weeks (emerging).
- **Work toward a weight that feels healthy for you.** Some research suggests that weight loss in women with a higher body weight may reduce hot flashes, though not for everyone.

> **TRY THIS WEEK**
>
> - Run your own version of Hannah's experiment: keep a trigger diary for two weeks, noting each hot flash and what you ate or drank in the hour before.
> - Swap your evening glass of wine for a Tart Cherry Evening Spritz or Cucumber-Mint Cooling Water on at least three nights.

Recipes to try: Turmeric Tofu & Veggie Scramble, Sesame Edamame Noodle Bowl, Miso-Glazed Tofu Buddha Bowl, Ginger-Garlic Tofu & Vegetable Stir-

Fry, Chili-Lime Edamame (go easy on the chili), Cucumber-Mint Cooling Water.

Sleep

Poor sleep is one of the most exhausting parts of the transition. Night sweats, anxiety, bathroom trips and lighter sleep all play a part, and your evening routine, including what and when you eat, can help.

Caffeine curfews, evening meals and sleepy foods

Caffeine can stay in your system for many hours, so try to have your last caffeinated drink before about 2 p.m., remembering that green and black tea, cola and dark chocolate contain it too. Alcohol may help you fall asleep, but it disrupts sleep later in the night and can trigger night sweats. A very large meal close to bedtime can keep you awake, and so can going to bed hungry, so finish your main meal two to three hours before bed and have a small snack later if you need one. Taper off fluids in the hour or two before bed to cut down on night-time bathroom trips, while still drinking well during the day.

Some foods may give sleep a gentle nudge. **Tryptophan,** an amino acid your body uses to make serotonin and melatonin, is found in turkey, chicken, fish, eggs, dairy, tofu, oats, nuts and seeds, and eating it with some carbohydrate may help it reach the brain. **Magnesium** plays a role in muscle relaxation and the nervous system; pumpkin seeds, almonds, spinach, black beans and whole grains are good sources, although evidence for magnesium supplements and sleep is limited. **Tart cherries** contain small amounts of natural melatonin, and a few small studies suggest tart cherry juice may modestly improve sleep in some adults (emerging). Choose unsweetened juice and keep the portion small.

Good bedtime snacks: a small pot of Greek yogurt with a sprinkle of oats, whole-grain toast with almond butter, a few walnuts with a small banana, a warm mug of milk or soy milk, or a caffeine-free Golden Turmeric Latte.

TRY THIS WEEK

- Set a 2 p.m. caffeine curfew for a week and note any change in your sleep.
- Try a small, calming bedtime snack instead of going to bed hungry or overfull.

Recipes to try: Baked Cinnamon Pear with Walnuts, Vanilla Chia Pudding Cups, Tart Cherry Evening Spritz, Golden Turmeric Latte, Greek Turkey Meatballs with Tzatziki, Three-Bean Vegetable Chili.

Mood and Anxiety

Mood changes are very common during the transition (Chapter 5). Food can't replace professional support when you need it, but it can make a meaningful difference to how resilient you feel from day to day.

Feeding a steadier mind

Long gaps between meals, or meals built mostly on refined carbohydrates, can lead to blood-sugar dips that show up as irritability, anxiety and low mood, so regular anchored meals are your first line of defense. Omega-3 fats, particularly EPA, are important for brain health, and some studies suggest they may help with depressive symptoms (emerging), so aim for oily fish twice a week or plant sources such as walnuts, chia and ground flaxseed.

Your gut matters here too. The gut and brain talk constantly through nerves, hormones and the immune system, a connection known as the **gut-brain axis**, and most of the body's serotonin is actually made in the gut. A varied, fiber-rich diet with fermented foods supports a healthy microbiome, which may in turn support mood (emerging). Several studies have linked Mediterranean-style eating with a lower risk of depression, and one well-known trial found that improving diet quality helped people with depression feel better (emerging). Low levels of folate, B12 and vitamin D have also been linked to low mood. And be mindful of alcohol, which is a depressant, and of too much caffeine, which can heighten anxiety and trigger palpitations.

TRY THIS WEEK

- Make sure you never go more than four or five hours without eating during the day this week.
- Add a portion of oily fish, or a tablespoon of ground flaxseed or chia seeds, on three days.

Recipes to try: Sheet-Pan Salmon with Broccoli & Sweet Potato, Smoked Salmon & Avocado Rye Toast, Golden Flax & Berry Overnight Oats, Walnut-Date Energy Bites, Kefir Berry Shake.

Energy and Fatigue

Feeling drained is one of the most common complaints in midlife. Poor sleep, stress, heavy periods and blood-sugar swings all play a part. If you're exhausted all the time, see your doctor to rule out anemia, thyroid problems, low vitamin D or B12, diabetes and depression.

Fueling a full day

Start with protein. A breakfast anchored with 25 to 30 grams keeps energy steady through the morning and cuts mid-morning cravings, as Keisha discovered in Chapter 9. If your periods are heavy, you may be low in iron even before you're anemic, so ask your doctor to check your ferritin, include iron-rich foods such as lean red meat, chicken, fish, lentils, beans, tofu and fortified cereals, and pair plant sources with vitamin C. Vitamin B12 absorption declines with age and with some medicines, and vegans need a reliable source, such as fortified foods or a supplement. Drink enough, because even mild dehydration makes you tired and foggy. Choose slow-release carbohydrates like whole grains, beans, lentils and starchy vegetables over white bread, pastries and sweets. And move: it sounds backward, but regular activity is one of the best energy boosters there is.

TRY THIS WEEK

- Anchor your breakfast with protein every day this week.
- Fill a water bottle each morning and aim to finish it by mid-afternoon.

Recipes to try: Spinach & Feta Egg Muffins, Besan Chilla with Mint Yogurt, Green Glow Protein Smoothie, Spiced Red Lentil Dal with Spinach, Lentil Bolognese with Whole-Grain Pasta.

Cravings and Emotional Eating

Many women notice stronger cravings during the transition, especially for sweet or starchy foods in the afternoon and evening.

Why the 4 p.m. cookie calls your name

Cravings have real causes. Blood sugar dips after meals low in protein and fiber. Poor sleep raises the hunger hormone ghrelin and lowers the fullness hormone leptin. Stress drives appetite through cortisol. Hormonal shifts, including premenstrual changes in perimenopause, play a part. Restriction backfires, because skipping meals or banning foods makes cravings stronger. And emotions such as boredom, loneliness, sadness and exhaustion often show up as hunger for comfort.

Kinder ways to handle them

- **Eat regular, satisfying meals.** Cravings are much easier to manage when you're not overly hungry.
- **Plan the snack.** If you always want something sweet at 4 p.m., plan for it: an apple with almond butter, a few squares of dark chocolate with almonds, or a yogurt bowl.
- **Pause and ask.** "Am I physically hungry, or is something else going on?" If it's something else, what do you really need? Rest, a break, connection, fresh air?
- **Allow all foods.** Labeling foods "forbidden" makes them more tempting. Enjoy treats slowly, without guilt.
- **Set up your environment.** Keep satisfying snacks easy to reach, and less nourishing ones a little less convenient.
- **Protect your sleep.** It's one of the most powerful craving-busters there is.

If emotional eating feels out of control, or food and eating cause you real distress, talk to your doctor or a registered dietitian. You deserve support.

TRY THIS WEEK

- Plan your afternoon snack in advance on five days this week.
- When a craving hits, wait ten minutes and drink a glass of water. If you still want the food, enjoy it slowly and without guilt.

Recipes to try: Dark Chocolate & Almond Clusters, Apple Slices with Almond Butter & Cinnamon, Berry Greek Yogurt Bark, Banana-Oat Mini Muffins, Spiced Pumpkin Seed Trail Mix.

Brain Fog

Forgetfulness, trouble concentrating and losing your train of thought are very common in perimenopause (Chapter 5), and they usually improve with time. Food can support your brain along the way. Omega-3 fats are a major structural part of the brain, so keep up the oily fish or the walnuts, chia and flaxseed. Steady blood sugar matters because your brain runs on a constant supply of glucose, and dips make concentration harder. Hydration matters because even mild dehydration affects attention and memory. Berries and leafy greens are rich in protective plant compounds, and the MIND diet, a blend of the Mediterranean and DASH diets that emphasizes them, has been linked with slower cognitive decline in observational studies (emerging). And sleep is when your brain clears waste and files away memories, which makes protecting it one of the best things you can do for brain fog.

TRY THIS WEEK

- Add a handful of berries and a portion of leafy greens to every day this week.

Recipes to try: Golden Flax & Berry Overnight Oats, Mediterranean Lentil & Roasted Vegetable Salad, Tuna & White Bean Niçoise Salad, Sheet-Pan Salmon with Broccoli & Sweet Potato, Frozen Yogurt-Dipped Blueberries.

Bloating and Digestion

Fluctuating hormones affect the gut, so bloating, gas, constipation and a more sensitive stomach are common in the transition, and stress and changes in eating patterns can add to it.

Calming an unsettled gut

- **Increase fiber gradually.** Fiber is essential, but adding a lot at once causes bloating. Build up over a few weeks and drink plenty of water so fiber can do its job.
- **Include fermented foods.** Yogurt, kefir, sauerkraut, kimchi, miso and tempeh contain live bacteria that may support gut health. Start small if they're new to you.
- **Slow down.** Eating quickly means swallowing more air, so eat slowly and chew well.
- **Walk after meals.** A short walk helps digestion.
- **Notice common triggers.** Fizzy drinks, chewing gum, sugar alcohols (like sorbitol and xylitol in "sugar-free" products), very fatty meals, large amounts of beans or cruciferous vegetables if you're not used to them, alcohol and caffeine all cause bloating in some people.
- **Get help with suspected sensitivities.** Some people react to fermentable carbohydrates known as FODMAPs (in foods like onion, garlic, wheat, some fruits and legumes). If you suspect this, work with a dietitian rather than cutting out lots of foods on your own.
- **Drink enough** to help prevent constipation.

> **WHEN TO SEE A DOCTOR**
>
> See your doctor if bloating is persistent (most days for three weeks or more), especially with loss of appetite, feeling full quickly, pain in your abdomen or pelvis, needing to urinate more often, changes in bowel habits, unexplained weight loss or bleeding. It's usually caused by something harmless, but persistent bloating can occasionally be a sign of ovarian or bowel cancer and should always be checked.

> **TRY THIS WEEK**
>
> - Add one small serving of a fermented food each day this week.
> - Take a 10-minute walk after your evening meal.

Recipes to try: Ginger Brown Rice Congee with Soft Egg, Greek Yogurt Power Bowl, Miso-Glazed Tofu Buddha Bowl, Kefir Berry Shake, Golden Turmeric Latte.

Joint Aches and Stiffness

Estrogen helps calm inflammation and maintain the tissues around your joints, so as it falls, many women notice new aches and stiffness, especially in the morning. An anti-inflammatory way of eating (Chapter 9), rich in vegetables, fruit, whole grains, beans, nuts, seeds, olive oil and oily fish, is a good foundation, and omega-3s deserve a regular place on your plate. Turmeric and ginger contain natural anti-inflammatory compounds. Research on turmeric (curcumin) supplements for joint pain is promising (emerging), but supplements can interact with some medicines, including blood thinners, so check with your doctor first; cooking with these spices generously is a safe, delicious start. Low vitamin D is common and can add to aches and muscle weakness, so it's worth checking. Keep moving, because walking, swimming, cycling, yoga and strength training keep joints mobile and build the muscles that support them. And if you're carrying extra weight, even small changes ease the load: every extra pound adds several pounds of pressure on your knees with each step.

> **TRY THIS WEEK**
>
> - Cook with turmeric, ginger or both at least three times this week.
> - Start each morning with five minutes of stretching from the Pose Library in Part Six.

Recipes to try: Turmeric Tofu & Veggie Scramble, Coconut Chickpea & Vegetable Curry, Spiced Red Lentil Dal with Spinach, Baked Cod with Tomatoes, Olives & Capers, Golden Turmeric Latte.

Your Symptom Cheat Sheet

Symptom	Foods and habits that may help	Possible triggers to notice
Hot flashes and night sweats	Regular anchored meals, soy foods, flaxseed, cold water, cooling foods	Alcohol, caffeine, spicy food, hot drinks, large late meals
Sleep	Tryptophan-rich foods, magnesium-rich foods, small bedtime snack, tart cherries	Caffeine after 2 p.m., alcohol, large or spicy late meals
Mood and anxiety	Steady blood sugar, oily fish, fiber and fermented foods, Mediterranean pattern	Skipped meals, alcohol, too much caffeine
Energy and fatigue	Protein at breakfast, iron-rich foods with vitamin C, B12, fluids, whole grains	Sugary snacks, skipped meals, dehydration
Cravings	Regular meals with protein and fiber, planned snacks, good sleep	Restriction, long gaps between meals, poor sleep, stress
Brain fog	Omega-3s, berries, leafy greens, steady blood sugar, fluids	Dehydration, poor sleep, blood-sugar dips
Bloating and digestion	Gradual fiber increase, fermented foods, fluids, walking after meals	Fizzy drinks, sugar alcohols, eating quickly, sudden fiber increases
Joint aches	Anti-inflammatory foods, omega-3s, turmeric, ginger, vitamin D, movement	Highly processed foods, inactivity

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Symptoms are one reason women change how they eat. The other, for many of us, is the number on the scale. Let's talk about that honestly.

CHAPTER 11

Rethinking Weight in Midlife

Your metabolism barely changes between the ages of 20 and 60. So why do so many women gain weight at 50?

> *Gloria found the jeans at the back of her closet while looking for a scarf: her favorite pair, the ones she'd worn to her son's wedding three years earlier. She was fifty-two. She tried them on, and they wouldn't button. She stood in front of the mirror for a long time, hearing her mother's voice from decades ago, a moment on the lips, a lifetime on the hips, and feeling the familiar pull toward the next diet. She'd done them all: the cabbage soup, the shakes, the one where you didn't eat after six. Each had worked for a while. None had lasted. That night she wrote in her journal: I'm tired of fighting my body. What if I tried understanding it instead?*

Weight is one of the most emotionally charged subjects for women in midlife. Many of us find that our weight and shape change during the transition, sometimes quickly, even though we haven't changed how we eat or move. It can feel confusing, frustrating and deeply unfair.

I know that feeling well. In the first two years of my own transition I gained eleven pounds, almost all of it around my middle, and my first instinct was to eat less and less. It made me hungrier, crankier and, eventually, heavier.

This chapter takes a compassionate, realistic look at what's really happening and what genuinely helps. You won't find crash diets, calorie targets or before-and-after photos here. You'll find an approach focused on health, strength and feeling good in your body, whatever its size.

The Metabolism Myth

On average, women gain about 1.5 pounds (0.7 kg) a year in midlife, though some gain much more and some none at all. It's natural to blame a "slowing

metabolism." But as you saw in Chapter 3, a landmark study of more than 6,000 people found that once you account for body size, **metabolic rate stays remarkably steady from 20 to 60.** That's liberating, because it means midlife weight change isn't a fixed sentence. It's driven by things you can influence.

So what is changing? Mostly **body composition,** the balance of muscle and fat, and **where** fat is stored.

As estrogen falls, the body shifts fat storage from the hips and thighs toward the abdomen. Muscle mass declines with age, faster around menopause, and less muscle means your body burns a little less energy and handles blood sugar less smoothly. Insulin sensitivity can dip, making it easier to store fat and harder to keep energy steady. Poor sleep raises hunger hormones, lowers fullness hormones and sends us looking for quick energy. Chronic stress and higher cortisol increase appetite and favor fat around the middle. Life changes too: busier schedules, caring responsibilities, less time to cook and move, new jobs or no job. And some medicines, including certain antidepressants and steroids, can contribute.

In other words, weight change in midlife is driven by biology and circumstance. It isn't a character flaw, and it isn't a lack of willpower.

Why the Middle Changes Shape

Even women whose weight stays the same often notice their waistline thickening. Before menopause, estrogen encourages fat storage under the skin on the hips, buttocks and thighs (subcutaneous fat). After menopause, more fat is stored deep inside the abdomen, around the organs. This is **visceral fat.**

Visceral fat matters because it's metabolically active: it releases substances that can increase inflammation and insulin resistance, and it's linked to a higher risk of heart disease and type 2 diabetes. That's why waist measurement can be a useful health check. As a general guide, many health organizations link a waist above 35 inches (88 cm) in women with higher health risks, although healthy thresholds vary with ethnicity and frame.

The encouraging news is that visceral fat responds well to the habits in this book: regular activity, especially strength training; a fiber-rich, plant-

forward diet; enough protein; good sleep; less alcohol; and stress management. These can shrink visceral fat even when the scale barely moves.

Why Crash Diets Backfire Now

When the jeans stop buttoning, the instinct is often to eat far less. Unfortunately, very low-calorie diets tend to backfire, and in midlife the cost is higher. When you eat too little, your body breaks down muscle along with fat, and losing muscle is exactly what we're trying to avoid. Low intakes of protein, calcium and vitamin D can harm your bones. Severe restriction triggers powerful biological hunger signals and cravings, and your metabolism adapts slightly to the smaller food supply. Most people regain the weight, and repeated cycles of losing and regaining can leave you with less muscle and more fat than when you started, as Gloria knows all too well. And along the way, crash diets take a toll on your mood, your energy and your relationship with food.

A slower, sustainable approach works far better in the long run.

Fasting, Keto and Other Trends

You've probably heard intermittent fasting, keto, carb cycling and a dozen other approaches promoted for menopause.

Intermittent fasting, such as eating within an 8- to 10-hour window or the "5:2" pattern, helps some people eat less overall, but studies generally find it works about as well as other ways of cutting calories, not better (emerging). Some women find it easy; others find it increases hunger, irritability or poor sleep, or makes it harder to anchor each meal with enough protein. If you try it, keep your meals balanced and protein-rich.

Ketogenic and very low-carb diets can produce weight loss in the short term, but they're hard to sustain, often low in fiber, and can be high in saturated fat, which may raise LDL cholesterol. They also cut out beans, whole grains and fruit, foods with well-established benefits for the heart and gut.

"Menopause supplements" and detox teas for weight loss generally have little or no evidence behind them.

The best way of eating for you is one you can follow happily and consistently, with enough protein, fiber and nutrients, that supports your health beyond the scale. For most women, Anchor, Add, Adapt (Chapter 9) does exactly that.

If you're using or considering one of the newer weight-loss medications, see Chapter 6, which covers what they are, their side effects, and how to protect your muscle if you and your doctor choose that route.

What Actually Works

What genuinely helps women feel healthier and more comfortable in their bodies through midlife? The research keeps pointing to the same handful of habits:

- **Anchor every meal with protein,** around 25 to 30 grams. It keeps you full, steadies blood sugar and protects muscle.
- **Add plants and fiber at every meal.** They fill you up, support your gut and help reduce visceral fat.
- **Strength train twice a week.** More muscle means better body composition, blood sugar and metabolism, even if your weight doesn't change much.
- **Walk every day,** especially after meals, to help with blood-sugar control.
- **Protect your sleep,** which regulates hunger and fullness hormones.
- **Drink less alcohol.** It adds calories, increases appetite, disrupts sleep and adds to abdominal fat.
- **Cook more, buy less ultra-processed food.** In a carefully controlled study at the US National Institutes of Health, people ate about 500 more calories a day on an ultra-processed diet than on a minimally processed one.
- **Eat with attention:** without screens, slowly, noticing your hunger and fullness.
- **Aim for "most of the time,"** not "all of the time." Changes you can keep for years beat dramatic ones that last weeks.

Progress the Scale Can't Show

The scale tells you only one thing: your total body weight. It can't tell muscle from fat, and it swings from day to day with fluid, hormones, digestion and

salt. If you're strength training, you may be building muscle and losing fat while your weight stays put.

So look for other signs. How do your clothes fit, especially around the waist? What's your waist measurement, checked once a month? Are you stronger, carrying heavier bags, climbing stairs more easily, doing more repetitions? How's your energy through the day, your sleep, your mood and confidence? Have your hot flashes, joint aches or digestion improved? What do your blood pressure, cholesterol and blood sugar say? And how do you feel about food: calmer, more relaxed, more in tune with your body?

If you choose to weigh yourself, consider doing it no more than once a week, at the same time of day, and focus on the trend over several weeks rather than any single number. If weighing yourself causes distress, it's perfectly fine not to.

> **BODY RESPECT**
>
> Health is about much more than body size. People of every shape and size can improve their health with the habits in this book. Wherever you're starting from, your body deserves care, nourishment and respect today, not just when it looks a certain way.

> **TRY THIS WEEK**
>
> - Choose one "beyond the scale" measure to track this month: your waist, your energy, or how many times you can stand up from a chair in 30 seconds.
> - Eat one meal a day this week without screens, slowly, noticing the flavors and how full you feel.

CHAPTER SUMMARY

- Metabolism stays remarkably steady from 20 to 60. Midlife weight change is driven mostly by muscle loss, fat redistribution, sleep, stress and life circumstances.
- Fat tends to move to the abdomen after menopause. Visceral fat responds well to activity, fiber, protein, sleep and less alcohol.
- Crash diets cost you muscle and bone and rarely last. Sustainable change works better.
- Fasting and keto help some people but aren't better than other approaches. Choose a pattern you can enjoy for life.
- Measure progress in many ways beyond the scale: strength, energy, sleep, waist, mood and health markers.

• • •

Gloria stopped fighting her body and started feeding it. The next chapter is about making that easy on even your busiest weeks.

CHAPTER 12

The Busy Woman's Kitchen

What would change if you never again had to decide what's for dinner at 6 p.m.?

> *Fatima used to dread Sunday evenings, when the week ahead loomed and the fridge held half an onion and a jar of olives. At forty-seven, with two jobs' worth of work squeezed into one, a son who ate like a linebacker and a husband who worked late, dinner had become a nightly scramble that usually ended in takeout. Then a friend talked her into trying "the Sunday hour." Now, while the kettle boils for her tea, she puts two trays of vegetables in the oven, starts a pot of lentils and boils a dozen eggs. By the time her tea's gone cold, the week is half cooked. "I didn't get more time," she says. "I just stopped wasting the time I had."*

You now know what your body needs. This chapter is about making it happen on real weekdays, when you're tired, short on time and juggling a dozen other things. The goal isn't to spend more time in the kitchen. It's to spend the time you have more wisely, the way Fatima does.

Twenty Minutes to a Plan

A weekly plan is one of the most powerful tools for eating well. It saves time, money and stress, and it means you're far less likely to end up eating whatever's fastest when you're exhausted. This routine takes about 20 minutes:

1. **Look at your week (2 minutes).** Which evenings are busy? When will you have time to cook? Any nights out?
2. **Pick your dinners (5 minutes).** Choose three or four main recipes, including at least one batch-cook or slow-cooker meal that makes leftovers. Plan one "pantry night" with something quick, and leave one night flexible.

3. **Turn leftovers into lunches (3 minutes).** Most dinners in this book become the next day's lunch, as they are or with a simple twist.
4. **Choose two breakfasts (3 minutes):** one make-ahead (like overnight oats or egg muffins) and one quick option for variety.
5. **Pick two or three snacks (2 minutes).**
6. **Write your grocery list (5 minutes).** Check what you already have, then list what you need by store section to save time in the aisles.

Part Five includes a ready-made 12-week meal plan with grocery lists, so you can skip the planning altogether if you prefer, and Appendix B has a blank planner for weeks when you want to make your own.

Cook Once, Eat Three Times

This is the secret to eating well without cooking every night: one batch of food becomes three different meals.

Cook once	Eat it again	And again
Sheet-Pan Salmon with Broccoli & Sweet Potato (dinner)	Flaked into a grain bowl with leafy greens (lunch)	Mashed with Greek yogurt and herbs on rye crackers (snack or light lunch)
Three-Bean Vegetable Chili (dinner)	Spooned over a baked sweet potato (lunch)	Wrapped in a whole-grain tortilla with avocado (quick dinner)
Lemon Herb Chicken Grain Bowl (lunch)	Extra chicken in a wrap with hummus and salad (lunch)	Stirred into soup with white beans and kale (dinner)
Spiced Red Lentil Dal with Spinach (dinner)	With brown rice and yogurt (lunch)	Thinned with stock into a warming soup (lunch)

Your Grocery List, Aisle by Aisle

A well-stocked kitchen makes the healthy choice the easy one. Use this list as a template and adapt it to your tastes, budget and needs.

Produce

- Leafy greens: spinach, kale, arugula, mixed salad leaves, bok choy
- Cruciferous vegetables: broccoli, cauliflower, cabbage, Brussels sprouts

- Colorful vegetables: bell peppers, carrots, tomatoes, beets, zucchini, red onions
- Aromatics: garlic, ginger, onions, scallions, fresh herbs, lemons, limes
- Starchy vegetables: sweet potatoes, potatoes, squash, corn
- Fruit: berries, apples, pears, oranges, bananas, kiwi
- Avocados

Protein

- Fish: salmon, cod, trout; canned sardines, salmon and tuna
- Poultry: chicken breasts or thighs, lean ground turkey
- Eggs
- Tofu (calcium-set if possible), tempeh, edamame
- Lean red meat (now and then, if you eat it)

Dairy and alternatives

- Plain Greek yogurt, kefir, cottage cheese
- Milk, or fortified unsweetened soy milk (good for calcium and protein)
- Cheese, such as feta, Parmesan and ricotta, in moderate amounts

Pantry

- Whole grains: rolled oats, quinoa, brown rice, whole-grain pasta, barley, buckwheat
- Whole-grain or seeded rye bread, whole-grain tortillas, corn tortillas
- Canned beans, chickpeas and lentils; dried red lentils
- Canned tomatoes, tomato paste, low-sodium stock
- Nuts and seeds: almonds, walnuts, pumpkin seeds, chia seeds, ground flaxseed, sesame seeds
- Nut and seed butters (no added sugar)
- Extra virgin olive oil, toasted sesame oil
- Vinegars, low-sodium soy sauce or tamari, miso paste, Dijon mustard
- Spices: turmeric, cumin, cinnamon, paprika, chili flakes, garam masala, dried oregano
- Dark chocolate (70% or higher)

Freezer

- Frozen vegetables: spinach, peas, broccoli, stir-fry mixes, cauliflower rice
- Frozen berries and mango
- Frozen edamame
- Frozen fish fillets and shrimp

- Batch-cooked soups, stews, chili and dal
- Whole-grain bread (it toasts perfectly from frozen)

Dinner From an "Empty" Fridge

With a few basics in the cupboard, you can make a real meal in minutes even when the fridge looks bare. A **bean and greens skillet** comes together from canned white beans, frozen spinach, garlic, canned tomatoes and a drizzle of olive oil, topped with an egg or served with toast. **Speedy fried rice** uses frozen brown rice, frozen vegetables, eggs or tofu, tamari and sesame oil. **Pantry pasta** is whole-grain pasta with canned tuna or chickpeas, canned tomatoes, olives, capers and frozen peas. For a **loaded sweet potato,** microwave one and pile on canned black beans, salsa, Greek yogurt and avocado. And a carton of good vegetable soup becomes a meal with a can of lentils and a handful of frozen spinach stirred in.

Add First, Swap Second

Small changes add up. Instead of thinking about what to cut out, think about what you can add, and which swaps feel easy.

Instead of…	Try…	Why
White bread	Seeded rye or whole-grain bread	More fiber, steadier energy
Sugary breakfast cereal	Overnight oats with berries and seeds	Protein, fiber and longer-lasting fullness
Flavored, sweetened yogurt	Plain Greek yogurt with fruit and cinnamon	More protein, less added sugar
Chips	Crispy Roasted Chickpeas or a small handful of nuts	Protein, fiber, healthy fats
Sweetened soda	Sparkling water with citrus, berries or cucumber	No added sugar; helps hydration
Creamy salad dressing	Olive oil, lemon and herbs	Heart-healthy fats
White rice	Brown rice, quinoa, or half rice and half cauliflower rice	More fiber and nutrients

Instead of...	Try...	Why
Processed deli meats	Leftover roast chicken, canned salmon, hummus or egg	Less salt and processed meat
Evening glass of wine	Tart Cherry Evening Spritz or an alcohol-free alternative	Better sleep, fewer hot flashes
Afternoon cookie	Two squares of dark chocolate with almonds	Satisfies the sweet craving with added nutrients
Cream in coffee	Milk or fortified soy milk	Less saturated fat, more calcium

And the easy additions: a handful of spinach in eggs, sauces and smoothies; a spoonful of ground flaxseed on oats or yogurt; a can of beans in soups and stews; seeds on salads; an extra vegetable with dinner; a piece of fruit with lunch.

The Sunday Hour

A little preparation at the start of the week makes weekday eating far easier. You don't need a whole afternoon; 60 to 90 minutes on a Sunday, or whatever day suits you, is plenty. Each week of the meal plan in Part Five includes a prep plan, and this is the basic routine:

1. **Turn on the oven** and roast two trays of vegetables (peppers, zucchini, red onion, broccoli, sweet potato) with olive oil.
2. **Cook a pot of grains,** such as quinoa or brown rice, for the next three or four days.
3. **Prepare a protein:** bake chicken, make a tray of Spinach & Feta Egg Muffins, or marinate a block of tofu.
4. **Make one batch meal,** such as a soup, chili or dal, for lunches or a busy evening.
5. **Wash and chop** salad vegetables and store them in containers lined with paper towels.
6. **Make overnight oats** or chia pudding for two or three breakfasts.
7. **Shake up a jar of salad dressing.**
8. **Portion snacks,** like nuts, seeds or energy bites, into small containers.

Keeping it safe

Cool cooked food quickly and refrigerate it within two hours. Most cooked dishes keep for three to four days in the fridge, and most soups, stews and chilis keep for about three months in the freezer, so label and date everything. Reheat food until it's steaming hot all the way through, reheat rice only once, and defrost frozen food in the fridge overnight rather than on the counter.

Drinking Enough Without Thinking About It

Many of us find it hard to drink enough, especially on busy days. Start with a glass of water before your morning coffee or tea. Keep a reusable bottle with you and refill it through the day. Link drinking to habits you already have: a glass with every meal, after every bathroom trip, or every time you check your email. Make water more interesting with citrus, cucumber, mint, berries or ginger. Herbal tea, milk, soy milk, sparkling water, and even tea and coffee all count, and so do water-rich foods like soup, cucumber, watermelon, oranges, berries and tomatoes. Drink extra after exercise, in hot weather and after hot flashes or night sweats.

Restaurants, Travel and Celebrations

Healthy eating should fit around your life, not the other way round. Social meals, celebrations and travel are part of a happy, connected life, and a few habits help you enjoy them and still feel good. Wherever you are, look for your anchor and your plants: a protein, plenty of vegetables and a fiber-rich carb, which nearly every menu offers. Don't arrive starving; a small snack beforehand makes choosing easier. Ask for what you need, like dressing on the side, extra vegetables or grilled instead of fried. Choose your treats: enjoy the dish or dessert you really want, slowly, and skip the ones you don't care about. Pace alcohol by alternating with water or trying alcohol-free options. When you travel, pack nuts, fruit, roasted chickpeas or protein bars, and look for a grocery store for easy meals. And let go of guilt. One meal or one weekend won't undo anything. Enjoy it, and pick up your usual routine at the next meal.

One Meal, Whole Family

You don't need to cook one meal for yourself and another for everyone else. The recipes in this book are family-friendly, and a few tricks help everyone enjoy the same dinner. Build-your-own meals like tacos, fajita bowls, grain bowls and stir-fries let each person choose their own toppings and portions. Adjust portions rather than recipes: growing teenagers or very active family members can take more grains and starchy vegetables while you prioritize your anchor and your plants. Put chili, hot sauce and strong spices on the table instead of in the pot, for those who like them (and for those whose hot flashes don't). Serve vegetables first, as a starter or side, when everyone's hungriest. And get the family involved, because children and partners who help choose and cook meals are far more likely to eat them.

Eating Well on a Budget

Nourishing food doesn't have to be expensive, and some of the healthiest foods are among the cheapest. Beans, lentils and chickpeas, canned or dried, are inexpensive protein and fiber, and dried are cheaper still. Frozen fruit and vegetables are just as nutritious as fresh, last for months and cut waste. Canned sardines, salmon and tuna provide protein and omega-3s, plus calcium if you eat the bones. Eggs and tofu are affordable, versatile anchors, and oats, brown rice and whole-grain pasta are cheap staples. Buy in season, choose store brands, and buy whole rather than prepared: a whole chicken, a bag of carrots or a block of cheese costs less than the pre-cut versions. Above all, plan and use your leftovers, because food waste is one of the biggest hidden costs in most households, and batch cooking and freezing portions helps with that too.

Making Any Recipe Work for You

Every recipe in this book includes labels and swaps so you can adapt it. These general swaps work across most of them:

If you need...	Swap...	For...
Gluten-free	Wheat pasta, bread, couscous, tortillas	Gluten-free pasta and bread, quinoa, rice, buckwheat, corn tortillas
Gluten-free	Soy sauce	Tamari or coconut aminos

If you need...	Swap...	For...
Gluten-free	Rolled oats	Certified gluten-free oats
Dairy-free	Milk	Fortified unsweetened soy, oat or almond milk
Dairy-free	Yogurt	Unsweetened soy or coconut yogurt (check for added calcium)
Dairy-free	Feta or Parmesan	Dairy-free alternatives, or nutritional yeast for a savory flavor
Vegetarian	Chicken, turkey or fish	Tofu, tempeh, beans, lentils or eggs
Vegan	Eggs (in baking)	1 tbsp ground flaxseed mixed with 3 tbsp water per egg
Vegan	Honey	Maple syrup or date syrup
Vegan	Fish sauce	Soy sauce or miso with a squeeze of lime

If you follow a vegan diet, pay particular attention to vitamin B12 (a supplement is essential), calcium, vitamin D, iron, iodine and omega-3s, and mix your protein sources every day. A registered dietitian can help you plan a balanced vegan diet.

> **TRY THIS WEEK**
>
> - Spend 20 minutes planning next week's meals using the steps above.
> - Try one Sunday hour, even if all you do is roast vegetables, cook a pot of grains and make one batch meal.

Your Kitchen Checklist

- ☐ I've planned my meals for the week.
- ☐ I have a grocery list organized by store section.
- ☐ My pantry has beans, lentils, whole grains, canned fish and tomatoes.
- ☐ My freezer has vegetables, fruit, edamame and a batch-cooked meal.
- ☐ I've done a prep session for the week ahead.
- ☐ My water bottle is ready for tomorrow.

☐ I have satisfying snacks prepared or within easy reach.

☐ I know which recipes I can adapt for my family.

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Your kitchen is ready. Now let's cook, starting with the meal that sets up your whole day.

PART FOUR

The Recipes

Thirty main recipes and twenty-five simple snacks, written for real kitchens, busy weeks and every level of cooking confidence.

CHAPTER 13

Breakfasts

Most women eat the least protein at breakfast, the meal where it helps the most.

> *For years my breakfast was a coffee and whatever my kids left on their plates. By ten I was ravenous, by eleven I was eating cookies from the office kitchen, and by three I was asleep on my feet. The day I started anchoring breakfast with protein (two eggs, a pot of yogurt, a tofu scramble) was the day my afternoons came back. That's why this chapter comes first.*

Breakfast is where Anchor, Add, Adapt makes its biggest difference. A morning meal with around 25 to 30 grams of protein, plenty of fiber and a little healthy fat keeps your blood sugar steady, quiets the mid-morning cravings and starts protecting your muscles before the day has properly begun. Most of these recipes can be made ahead, because nobody has time to cook at 7 a.m. on a Wednesday.

How the Recipes Work

Every recipe follows the same layout. The **labels** under each title tell you whether it's gluten-free (GF), dairy-free (DF), vegetarian (V) or vegan (VG), or can easily be made so ("option"). **Swaps & Variations** shows you how to adapt it to your needs or to what's in your fridge, and **Why It Helps** explains what it does for you during the transition.

A few kitchen notes

- 1 tablespoon (tbsp) = 15 ml; 1 teaspoon (tsp) = 5 ml; 1 cup = 240 ml.
- Oven temperatures are for conventional ovens; for fan ovens, reduce by about 25°F (15°C).
- Eggs are large, olive oil is extra virgin, and "milk" means any dairy or unsweetened, fortified plant milk.

- For gluten-free cooking, check that oats, soy sauce, miso, stock and spice mixes are certified gluten-free.

Golden Flax & Berry Overnight Oats

V · VG OPTION · GF OPTION · MAKE-AHEAD

SERVES	PREP	COOK	KEEPS
2	10 min	None	3 days

This is my answer to the 6:45 a.m. scramble. Five minutes the night before, while the kettle boils, and breakfast is waiting in the fridge like a favor from your past self: cool, creamy oats, berries bleeding purple into the yogurt, and a warm hint of cinnamon. Greek yogurt and soy milk do the anchoring, and ground flaxseed slips in fiber and omega-3s without you noticing.

Ingredients

- 1 cup (90 g) rolled oats
- 2 tbsp ground flaxseed
- 1 tbsp chia seeds
- 1/2 tsp ground cinnamon
- 1/4 tsp ground turmeric (optional, for a golden color)
- 1 cup (240 ml) unsweetened soy milk or milk of your choice
- 3/4 cup (170 g) plain Greek yogurt
- 1 to 2 tsp maple syrup or honey (optional)
- 1/2 tsp vanilla extract
- 1 cup (150 g) mixed berries, fresh or frozen
- 2 tbsp chopped walnuts or almonds, to serve

Method

1. In a bowl or large jar, stir together the oats, flaxseed, chia seeds, cinnamon and turmeric.
2. Add the milk, yogurt, maple syrup (if using) and vanilla. Stir well, making sure there are no dry pockets of oats.
3. Divide between two jars or containers. Top each with half the berries. If using frozen berries, they will thaw overnight and release their juices.
4. Cover and refrigerate for at least 4 hours, or overnight.
5. In the morning, stir, add a splash more milk if you like a looser texture, and top with the chopped nuts. Eat cold, or warm in the microwave for 1 to 2 minutes.

SWAPS & VARIATIONS

- **Vegan:** use soy yogurt and maple syrup.
- **Gluten-free:** use certified gluten-free oats.
- **More protein:** stir in an extra 1/4 cup (60 g) Greek yogurt or 1 tbsp of a protein powder you like.
- **Flavors:** try grated apple and cinnamon, mashed banana and peanut butter, or mango and lime zest.

WHY IT HELPS

Oats provide soluble fiber that supports healthy cholesterol and steady energy. Flaxseed adds lignans and plant omega-3s, and the combination of yogurt and soy milk gives you a protein-rich start that keeps you full until lunch.

Spinach & Feta Egg Muffins

GF · V · MAKE-AHEAD

MAKES	PREP	COOK	KEEPS
12	15 min	20 min	4 days

Make a tray on Sunday and you've solved four weekday breakfasts before Monday even starts. They're fluffy, salty little frittatas studded with sweet pepper and spinach, and they taste just as good cold from the fridge, eaten in the car, as they do warm on a plate. Three of them, with a piece of fruit, will carry you comfortably to lunch.

Ingredients

- Olive oil or cooking spray, for greasing
- 10 eggs
- 1/4 cup (60 ml) milk
- 1/4 tsp salt and plenty of black pepper
- 2 cups (60 g) baby spinach, chopped
- 1 small red bell pepper, finely diced
- 3 scallions, thinly sliced
- 1/2 cup (75 g) crumbled feta cheese
- 1/2 cup (75 g) cherry tomatoes, quartered
- 1 tsp dried oregano

Method

1. Heat the oven to 350°F (180°C). Lightly grease a 12-cup muffin tin with olive oil, or line it with silicone or parchment muffin cases.
2. In a large jug or bowl, whisk the eggs with the milk, salt and pepper.
3. Divide the spinach, red pepper, scallions, feta and tomatoes evenly between the muffin cups. Sprinkle with the oregano.
4. Pour the egg mixture over the vegetables, filling each cup about three-quarters full.
5. Bake for 18 to 22 minutes, until puffed, golden and just set in the center (a knife inserted should come out clean).
6. Cool in the tin for 5 minutes, then run a knife around the edges and lift out.

SWAPS & VARIATIONS

- **Dairy-free:** use a plant milk and replace the feta with diced avocado added after baking, or a dairy-free cheese.
- **Vegetables:** use any leftover roasted vegetables, mushrooms, zucchini or kale.
- **Make it a meal:** serve with a slice of whole-grain toast and some fruit.
- **Storage:** keep in an airtight container in the fridge for up to 4 days, or freeze for up to 2 months. Reheat in the microwave for 30 to 45 seconds.

WHY IT HELPS

Eggs provide high-quality protein, vitamin B12 and some vitamin D. Feta adds calcium, and spinach contributes folate and magnesium. Having a protein-rich breakfast ready to go makes it much easier to avoid mid-morning energy crashes.

Greek Yogurt Power Bowl with Walnuts & Seeds

GF · V · 10 MIN

SERVES	PREP	COOK	BEST
1	10 min	None	fresh

On the mornings when cooking is out of the question, this is the bowl I build standing at the counter: thick, tangy yogurt, whatever fruit is ripe, and a shower of crunchy seeds and nuts. It takes less time than waiting in line for a latte, and it keeps you full far longer. Think of it as a template and change the toppings with the seasons.

Ingredients

- 1 cup (225 g) plain Greek yogurt
- 1/2 tsp vanilla extract (optional)
- 1/2 cup (75 g) berries or chopped fruit, such as kiwi, mango or pear
- 1 tbsp ground flaxseed
- 1 tbsp pumpkin seeds
- 1 tbsp chopped walnuts
- 2 tbsp low-sugar granola or toasted rolled oats
- A pinch of ground cinnamon
- 1 tsp honey or maple syrup (optional)

Method

1. Spoon the yogurt into a bowl and stir in the vanilla, if using.
2. Top with the fruit, flaxseed, pumpkin seeds, walnuts and granola.
3. Finish with a pinch of cinnamon and a drizzle of honey, if you like. Eat straight away so the toppings stay crunchy.

SWAPS & VARIATIONS

- **Dairy-free / vegan:** use unsweetened soy yogurt (it has the most protein of the plant yogurts) and maple syrup.
- **Gluten-free:** check that your granola or oats are certified gluten-free.
- **Gut boost:** use half kefir and half yogurt for extra live cultures.
- **To go:** layer in a jar with the toppings in a separate small container.

WHY IT HELPS

Greek yogurt is one of the easiest ways to get both protein and calcium at breakfast. Walnuts and flaxseed provide plant omega-3s, pumpkin seeds add magnesium, and fruit adds fiber and antioxidants.

Turmeric Tofu & Veggie Scramble

GF · DF · VG

SERVES	PREP	COOK	KEEPS
2	10 min	12 min	2 days

If you've never tried a tofu scramble, this is the one to convert you. Crumbled tofu turns sunny gold with turmeric and soaks up cumin and smoky paprika, while the peppers and mushrooms bring color and bite. It's a plant-based plate that eats like a diner breakfast, and if you pick calcium-set tofu, it's a calcium boost too.

Ingredients

- 1 tbsp olive oil
- 1/2 small red onion, finely chopped
- 1 small red bell pepper, diced
- 1 cup (70 g) mushrooms, sliced
- 1 block (14 oz / 400 g) firm tofu, drained and patted dry
- 1/2 tsp ground turmeric
- 1/2 tsp ground cumin
- 1/4 tsp smoked paprika
- 1 tbsp nutritional yeast (optional)
- 2 tbsp unsweetened soy milk or water
- 2 large handfuls (60 g) baby spinach
- Salt and black pepper, to taste
- Fresh herbs, such as cilantro or chives, to serve
- Whole-grain or gluten-free toast, to serve (optional)

Method

1. Heat the olive oil in a large nonstick frying pan over medium heat. Add the onion and cook for 3 minutes until softening.
2. Add the red pepper and mushrooms and cook for 4 to 5 minutes, stirring now and then, until tender.
3. Crumble the tofu into the pan with your hands, in pieces about the size of scrambled eggs.
4. Sprinkle over the turmeric, cumin, paprika and nutritional yeast (if using). Add the soy milk and stir well to coat the tofu evenly.
5. Cook for 3 to 4 minutes, stirring gently, until the tofu is heated through.

6. Add the spinach and stir until just wilted. Season with salt and pepper.
7. Serve topped with fresh herbs, with toast on the side if you like.

SWAPS & VARIATIONS

- **Calcium boost:** choose tofu labeled "calcium-set" or "made with calcium sulfate."
- **Eggy flavor:** a pinch of kala namak (Indian black salt) at the end gives a surprisingly egg-like taste.
- **Make it a wrap:** spoon into a whole-grain or gluten-free tortilla with avocado and salsa.
- **Spice level:** add chili flakes if you like heat, or leave out if spicy food triggers hot flashes.

WHY IT HELPS

Tofu provides protein and soy isoflavones, which may modestly reduce hot flashes for some women. Turmeric adds anti-inflammatory compounds, and the vegetables contribute fiber, folate and antioxidants.

Besan Chilla with Mint Yogurt

GF · V · DF OPTION · VG OPTION

SERVES	PREP	COOK	BEST
2	10 min	15 min	fresh

Across North India, these savory chickpea-flour pancakes are the breakfast you make when there's "nothing in the house." The batter takes five minutes, the edges crisp up in the pan, and every bite is flecked with onion, tomato, ginger and cilantro. Served with a cool swirl of mint yogurt, they're naturally gluten-free and full of plant protein. They also make a brilliant light lunch.

Ingredients

For the chilla (makes 4)

- 1 cup (100 g) chickpea flour (besan or gram flour)
- 1/2 tsp ground cumin
- 1/4 tsp ground turmeric
- 1/4 tsp salt
- About 3/4 cup (180 ml) water
- 1/2 small red onion, very finely chopped
- 1 small tomato, seeded and finely chopped
- 1 handful (15 g) spinach, finely chopped
- 2 tbsp chopped fresh cilantro
- 1/2 green chili, finely chopped (optional)
- 1/2 tsp grated fresh ginger
- 2 to 3 tsp olive or neutral oil, for cooking

For the mint yogurt

- 1 cup (225 g) plain Greek yogurt
- 2 tbsp chopped fresh mint
- A squeeze of lemon juice
- A pinch of salt and ground cumin

Method

1. In a bowl, whisk the chickpea flour, cumin, turmeric and salt. Gradually whisk in the water until you have a smooth batter about the thickness of heavy cream. Let it rest for 5 minutes.
2. Meanwhile, stir together all the mint yogurt ingredients and set aside.

3. Stir the onion, tomato, spinach, cilantro, chili (if using) and ginger into the batter.
4. Heat a nonstick frying pan over medium heat and brush with a little oil. Pour in a quarter of the batter and quickly spread it into a thin round, about 6 inches (15 cm) across, with the back of a ladle.
5. Cook for 2 to 3 minutes, until the edges look dry and the underside is golden. Drizzle a few drops of oil around the edge, flip, and cook for another 1 to 2 minutes.
6. Keep warm while you cook the rest, adding a little oil each time.
7. Serve the chilla warm with the mint yogurt.

SWAPS & VARIATIONS

- **Dairy-free / vegan:** make the mint yogurt with unsweetened soy or coconut yogurt.
- **More protein:** serve with a side of Greek yogurt, a boiled egg or crumbled paneer or tofu stirred into the batter.
- **Mild version:** leave out the chili, especially if spicy food triggers hot flashes.
- **Batter tip:** if the batter thickens as it stands, whisk in a splash more water.

WHY IT HELPS

Chickpea flour provides plant protein, fiber, folate, iron and magnesium, which help keep energy steady. The yogurt adds protein and calcium, and the vegetables and herbs bring color, flavor and antioxidants.

Green Glow Protein Smoothie

GF · V · DF OPTION · 5 MIN

SERVES	PREP	COOK	BEST
1	5 min	None	fresh

For mornings when you wake up already too warm, this frosty, tropical smoothie is pure relief. Frozen mango and banana make it thick and cold, the spinach disappears completely (I promise), and Greek yogurt and nut butter make it a real meal rather than a sugary drink. Breakfast in a glass, in the time it takes to find your keys.

Ingredients

- 1 cup (240 ml) unsweetened soy milk or milk
- 1/2 cup (115 g) plain Greek yogurt
- 1 large handful (30 g) baby spinach
- 1/2 small frozen banana
- 1/2 cup (75 g) frozen mango or pineapple chunks
- 1 tbsp ground flaxseed
- 1 tbsp almond or peanut butter
- A small piece of fresh ginger (optional)
- Ice cubes (optional)

Method

1. Add the milk to a blender first, then the yogurt, spinach, banana, mango, flaxseed, nut butter and ginger (if using).
2. Blend on high for 45 to 60 seconds, until completely smooth. Add a few ice cubes for a thicker, colder smoothie, or a splash more milk to thin it.
3. Pour into a glass and drink straight away.

SWAPS & VARIATIONS

- **Dairy-free:** use soy yogurt or a scoop of a plant-based protein powder instead of Greek yogurt.
- **Lower sugar:** replace the banana with 1/2 cup (75 g) frozen cauliflower rice. It adds creaminess without changing the flavor.
- **Prep ahead:** portion the spinach, fruit and flaxseed into freezer bags so you just add milk, yogurt and nut butter in the morning.

WHY IT HELPS

Soy milk and Greek yogurt together provide around 20 grams of protein, plus calcium. Spinach adds folate and magnesium, flaxseed adds fiber and omega-3s, and nut butter brings healthy fats to keep you full.

Smoked Salmon & Avocado Rye Toast

DF · GF OPTION

SERVES	PREP	COOK	BEST
2	10 min	3 min	fresh

This is my Saturday-morning treat that's quick enough for a Tuesday. Dark, chewy rye, buttery avocado mashed with lemon, silky salmon and a scatter of dill and capers: it feels like brunch at a café, and it's on the table in ten minutes. Salmon brings omega-3s and vitamin D; rye brings the slow, steady energy.

Ingredients

- 4 slices seeded rye or whole-grain bread
- 1 ripe avocado
- Juice of 1/2 lemon
- A pinch of chili flakes (optional)
- 4 oz (120 g) smoked salmon
- 1/4 cucumber, thinly sliced
- 1 tbsp capers, drained (optional)
- 1 tbsp chopped fresh dill or chives
- Black pepper
- Lemon wedges, to serve

Method

1. Toast the bread.
2. Meanwhile, halve and pit the avocado, scoop it into a bowl and mash with the lemon juice, a pinch of black pepper and chili flakes (if using).
3. Spread the avocado over the toast.
4. Top with the cucumber slices and smoked salmon.
5. Scatter over the capers and fresh herbs, add a twist of black pepper and serve with lemon wedges.

SWAPS & VARIATIONS

- **Gluten-free:** use a seeded gluten-free bread.
- **No fish:** top with a jammy boiled egg, smoked tofu or a spoonful of white beans mashed with lemon.
- **Budget swap:** use canned salmon or sardines mashed with lemon.
- **Salt watch:** smoked salmon is quite salty, so there is no need to add extra salt.

WHY IT HELPS

Salmon is one of the best sources of omega-3 fats and also provides vitamin D and protein. Avocado adds healthy fats and fiber, and rye bread offers steadier energy than white bread.

Ginger Brown Rice Congee with Soft Egg

GF · DF · V · MAKE-AHEAD

SERVES	PREP	COOK	KEEPS
4	10 min	1 hr 15 min	4 days

Congee is the bowl that people across East Asia turn to when they need comfort, and on a morning when your stomach feels unsettled or you've slept badly, nothing soothes quite like it. Brown rice simmers slowly with ginger and garlic until it collapses into a silky porridge, then gets crowned with a jammy egg, scallions and a few drops of sesame oil. Make a big pot; it keeps for days.

Ingredients

- 3/4 cup (150 g) brown rice, rinsed
- 7 cups (1.7 liters) low-sodium vegetable stock or water, plus extra as needed
- A thumb-sized piece (2 in / 5 cm) of fresh ginger, peeled and cut into thin matchsticks
- 2 garlic cloves, lightly crushed
- 1 cup (70 g) shiitake or other mushrooms, sliced
- 2 cups (140 g) chopped bok choy or spinach
- 4 eggs
- 2 tsp tamari or low-sodium soy sauce
- 1 tsp toasted sesame oil
- 2 scallions, thinly sliced
- 1 tbsp toasted sesame seeds
- Chili crisp or sliced fresh chili, to serve (optional)

Method

1. Put the rice, stock, ginger and garlic in a large saucepan. Bring to a boil, then reduce the heat to low, partly cover and simmer for 60 to 70 minutes, stirring every 10 to 15 minutes, until the rice has broken down into a thick, creamy porridge. Add more stock or water if it becomes too thick.
2. Remove the garlic. Stir in the mushrooms and cook for 5 minutes, then add the bok choy and cook for 2 to 3 minutes more, until just tender.

3. Meanwhile, bring a small pan of water to a boil. Gently lower in the eggs and cook for 6 1/2 minutes for soft, jammy yolks. Transfer to cold water, then peel.
4. Season the congee with the tamari. Ladle into bowls and top each with a halved egg, a few drops of sesame oil, scallions and sesame seeds. Add chili if you like.

SWAPS & VARIATIONS

- **Faster:** cook the congee in a pressure cooker or multicooker on the porridge setting (around 30 minutes), or use leftover cooked brown rice simmered with stock for 20 to 25 minutes.
- **More protein:** add shredded leftover chicken or cubes of tofu with the mushrooms.
- **Vegan:** replace the egg with cubes of silken or firm tofu.
- **Gluten-free:** use tamari labeled gluten-free.
- **Make ahead:** congee thickens as it cools. Reheat with a splash of water or stock, and cook the eggs fresh.

WHY IT HELPS

Brown rice provides fiber and B vitamins, and the soft, warm texture is easy to digest. Ginger has long been used to settle the stomach, and the eggs add protein and vitamin B12.

CHAPTER 14

Lunches

The 3 p.m. slump usually starts at noon, with what's in your lunchbox.

> *Dana, a fifty-year-old paralegal, used to eat lunch at her desk: a granola bar and a diet soda, or nothing at all if the morning ran long. By mid-afternoon she'd be reading the same paragraph three times, then raiding the candy jar by the printer. When she started packing leftovers from dinner (a scoop of dal with rice, a grain bowl with chicken, a jar of soup) her afternoons changed. "It's the same job," she says. "I'm just not running on fumes anymore."*

A good lunch carries you through the afternoon without the slump. These recipes are built on Anchor, Add, Adapt: a generous protein anchor, plenty of plants, a fiber-rich carbohydrate and a little healthy fat. Most of them pack well, keep for several days in the fridge and work just as happily as a light dinner. Many also fit the "cook once, eat three times" approach, so a pot of soup, dal or grain salad on Sunday can cover lunch for much of the week. Look for the **Make-Ahead** label and the storage notes in each recipe.

PACKING TIPS

- Keep dressings and crunchy toppings in a small separate container and add them just before eating.
- Pack soups and dal piping hot in an insulated food flask to keep them warm until lunchtime.
- Add a piece of fruit and a small handful of nuts if you have a long afternoon ahead.

Mediterranean Lentil & Roasted Vegetable Salad

GF · DF OPTION · VG OPTION · MAKE-AHEAD

SERVES	PREP	COOK	KEEPS
4	15 min	30 min	4 days

This is the salad I make when I know the week is going to be busy. It actually gets better after a night in the fridge, as the lentils soak up the lemony, garlicky dressing. Charred peppers and sweet roasted tomatoes, salty feta and olives, a crunch of pumpkin seeds: it's a lunch that makes your colleagues ask what you're eating.

Ingredients

- 1 medium zucchini, cut into chunks
- 1 red bell pepper, cut into chunks
- 1 small red onion, cut into wedges
- 1 1/2 cups (225 g) cherry tomatoes
- 2 tbsp olive oil
- 1 tsp dried oregano
- 2 cans (15 oz / 400 g each) green or brown lentils, drained and rinsed (or 3 cups / 600 g cooked)
- 2 large handfuls (60 g) arugula or baby spinach
- 1/2 cup (75 g) crumbled feta cheese
- 1/4 cup (35 g) pitted Kalamata olives, halved
- 2 tbsp toasted pumpkin seeds

For the dressing

- 3 tbsp olive oil
- Juice of 1 lemon
- 1 tsp Dijon mustard
- 1 small garlic clove, grated
- 2 tbsp chopped fresh parsley or mint
- Salt and black pepper

Method

1. Heat the oven to 425°F (220°C). Spread the zucchini, pepper, onion and tomatoes on a large baking tray. Toss with the olive oil, oregano, a pinch of salt and pepper.

2. Roast for 25 to 30 minutes, turning once, until tender and lightly charred.
3. Meanwhile, whisk all the dressing ingredients together in a large bowl.
4. Add the lentils to the dressing and toss well.
5. Add the roasted vegetables and arugula and toss gently.
6. Scatter over the feta, olives and pumpkin seeds. Serve warm or at room temperature.

SWAPS & VARIATIONS

- **Dairy-free / vegan:** leave out the feta, or use a dairy-free alternative, and add cubes of baked tofu or extra chickpeas for protein.
- **More protein:** top with flaked salmon, grilled chicken or a boiled egg.
- **Grains:** stir in 1 cup (185 g) cooked quinoa or farro (farro isn't gluten-free).
- **Storage:** keep the arugula separate if making ahead, and add just before eating.

WHY IT HELPS

Lentils provide plant protein, fiber, folate and iron. The colorful vegetables bring antioxidants, olive oil adds heart-healthy fats, and feta contributes calcium.

Sesame Edamame Noodle Bowl

DF · VG · GF OPTION · 30 MIN

SERVES	PREP	COOK	KEEPS
2	15 min	10 min	3 days

Cold noodles might sound like an odd lunch until you taste these: slippery soba tossed in a nutty sesame-ginger dressing, with crisp ribbons of carrot, cabbage and cucumber and bright green edamame. It's cool, crunchy and deeply satisfying on a warm day, and it packs beautifully in a lunchbox.

Ingredients

- 4 oz (115 g) soba (buckwheat) noodles or whole-wheat noodles
- 1 cup (150 g) frozen shelled edamame
- 1 carrot, cut into matchsticks or grated
- 1 small red bell pepper, thinly sliced
- 1 cup (70 g) red cabbage, finely shredded
- 1/4 cucumber, cut into matchsticks
- 2 scallions, thinly sliced
- 2 tbsp chopped fresh cilantro
- 1 tbsp toasted sesame seeds

For the dressing

- 2 tbsp tahini or natural peanut butter
- 2 tbsp low-sodium soy sauce or tamari
- 1 tbsp rice vinegar
- 1 tsp toasted sesame oil
- 1 tsp maple syrup
- 1 tsp grated fresh ginger
- 1 small garlic clove, grated
- 2 to 3 tbsp warm water

Method

1. Cook the noodles according to the package instructions, adding the edamame for the last 3 minutes. Drain, rinse under cold water and drain well.
2. Whisk all the dressing ingredients together, adding enough warm water to make a smooth, pourable dressing.

3. In a large bowl, toss the noodles and edamame with the carrot, pepper, cabbage, cucumber and most of the scallions.
4. Pour over the dressing and toss until everything is coated.
5. Divide between bowls or containers and top with the remaining scallions, cilantro and sesame seeds.

SWAPS & VARIATIONS

- **Gluten-free:** use 100% buckwheat soba or rice noodles, and tamari instead of soy sauce.
- **More protein:** add cubes of baked tofu, shredded chicken or cooked shrimp.
- **Lower carb:** replace half the noodles with spiralized zucchini or extra cabbage.
- **Storage:** soba can stick together after a day or two; loosen with a splash of water or extra dressing.

WHY IT HELPS

Edamame are young soybeans, rich in protein, fiber and isoflavones. Tahini adds calcium and healthy fats, and the raw vegetables provide fiber, vitamin C and crunch.

Black Bean, Corn & Avocado Tacos

GF · DF · VG · 30 MIN

SERVES	PREP	COOK	BEST
2 to 3	15 min	10 min	fresh

Inspired by Mexican street food, these are the tacos I make when lunch needs to feel like a small celebration. Smoky, cumin-spiced black beans, sweet corn, a sharp lime slaw and creamy avocado, all piled into warm corn tortillas. They're ready in the time it takes to warm the tortillas, and they're naturally gluten-free and vegan.

Ingredients

- 1 tbsp olive oil
- 1/2 red onion, finely chopped
- 1 garlic clove, crushed
- 1 tsp ground cumin
- 1 tsp smoked paprika
- 1 can (15 oz / 400 g) black beans, drained and rinsed
- 1 cup (150 g) corn kernels (frozen, canned or fresh)
- Juice of 1 lime, divided
- 6 small corn tortillas
- 1 ripe avocado, sliced
- 1/4 cup (60 g) salsa or pico de gallo
- Fresh cilantro, to serve

For the lime slaw

- 1 1/2 cups (100 g) finely shredded red and green cabbage
- Juice of 1/2 lime
- 1 tsp olive oil
- A pinch of salt

Method

1. Toss the slaw ingredients together in a bowl and set aside.
2. Heat the olive oil in a frying pan over medium heat. Add the onion and cook for 3 to 4 minutes until softened. Add the garlic, cumin and paprika and cook for 30 seconds.

3. Add the black beans and corn with 2 tbsp water. Cook for 4 to 5 minutes, lightly crushing some of the beans with a fork. Add half the lime juice and season to taste.
4. Warm the tortillas in a dry frying pan for 20 to 30 seconds per side, or wrap in a damp paper towel and microwave for 30 seconds.
5. Fill each tortilla with the bean mixture, slaw, avocado and salsa. Finish with cilantro and the remaining lime juice.

SWAPS & VARIATIONS

- **Gluten-free:** check that your corn tortillas are 100% corn and labeled gluten-free.
- **More protein:** add a spoonful of Greek yogurt (not vegan) or crumbled feta, or serve with a side of scrambled eggs.
- **Make it a bowl:** serve the beans and toppings over brown rice or quinoa for a packed lunch.
- **Mild:** choose a mild salsa if spicy food triggers hot flashes.

WHY IT HELPS

Black beans provide protein, fiber, folate and magnesium. Avocado adds healthy fats and fiber, cabbage brings vitamin C, and corn tortillas are a naturally whole-grain base.

Tuscan White Bean & Kale Soup

GF · DF · VG · MAKE-AHEAD

SERVES	PREP	COOK	KEEPS
4 to 6	15 min	35 min	5 days

Rustic, cheap and deeply comforting, this is the soup I make on a gray Sunday so the week has a head start. Mashing some of the beans in the pot gives it a creamy body without any cream, while rosemary, garlic and a squeeze of lemon keep it bright. It freezes beautifully, so make a big pot and stash portions for the days you can't face cooking.

Ingredients

- 2 tbsp olive oil
- 1 onion, finely chopped
- 2 carrots, diced
- 2 celery stalks, diced
- 3 garlic cloves, crushed
- 1 tsp dried rosemary or 1 sprig fresh
- 1/2 tsp dried thyme
- 1/4 tsp chili flakes (optional)
- 1 can (14 oz / 400 g) chopped tomatoes
- 5 cups (1.2 liters) low-sodium vegetable stock
- 2 cans (15 oz / 400 g each) cannellini or other white beans, drained and rinsed
- 4 cups (150 g) chopped kale, tough stems removed
- Juice of 1/2 lemon
- Salt and black pepper
- Grated Parmesan, to serve (optional)

Method

1. Heat the olive oil in a large pot over medium heat. Add the onion, carrots and celery and cook for 8 to 10 minutes, stirring occasionally, until softened.
2. Add the garlic, rosemary, thyme and chili flakes (if using) and cook for 1 minute.
3. Add the tomatoes and stock and bring to a boil.

4. Add the beans. Use a potato masher or fork to lightly mash about a quarter of them in the pot, which helps thicken the soup. Reduce the heat and simmer for 15 minutes.
5. Stir in the kale and simmer for 5 to 7 minutes more, until tender.
6. Add the lemon juice and season to taste. Serve topped with a little Parmesan, if you like.

SWAPS & VARIATIONS

- **More protein:** add cooked shredded chicken, or serve with a slice of seeded rye toast topped with ricotta (not vegan).
- **Greens:** swap the kale for spinach, Swiss chard or cavolo nero.
- **Gluten-free:** check that your stock is gluten-free.
- **Freezing:** cool quickly and freeze in portions for up to 3 months. Defrost overnight in the fridge and reheat until steaming hot.

WHY IT HELPS

White beans are an excellent source of plant protein, fiber and potassium. Kale provides calcium, vitamin K and folate, and the soup's high water content helps hydration.

Tuna & White Bean Niçoise Salad

GF · DF

SERVES	PREP	COOK	KEEPS
2	15 min	12 min	2 days

The classic French salad, reworked for a hungry midweek lunch. Creamy white beans join the new potatoes for extra fiber, a jammy egg and good canned tuna do the anchoring, and a sharp mustard vinaigrette pulls it all together. It looks elegant enough for guests and costs very little to make.

Ingredients

- 8 oz (225 g) small new potatoes, halved
- 1 cup (100 g) green beans, trimmed
- 2 eggs
- 1 can (5 oz / 140 g) tuna in olive oil or water, drained
- 1 can (15 oz / 400 g) cannellini beans, drained and rinsed
- 1 cup (150 g) cherry tomatoes, halved
- 1 head little gem or romaine lettuce, leaves separated
- 1/4 cup (35 g) black olives
- 1 tbsp capers (optional)

For the dressing

- 3 tbsp olive oil
- 1 tbsp red wine vinegar
- 1 tsp Dijon mustard
- 1 tsp chopped fresh chives or parsley
- Salt and black pepper

Method

1. Put the potatoes in a pan of cold water, bring to a boil and cook for 10 to 12 minutes until tender. Add the green beans for the last 3 minutes. Drain and cool under cold water.
2. Meanwhile, bring a small pan of water to a boil. Gently lower in the eggs and cook for 7 to 8 minutes. Cool in cold water, then peel and halve.
3. Whisk the dressing ingredients together.
4. Arrange the lettuce on plates or in containers. Top with the potatoes, green beans, white beans, tomatoes, tuna, olives and capers.
5. Add the eggs and drizzle with the dressing just before serving.

SWAPS & VARIATIONS

- **Fish swap:** use canned salmon, sardines or leftover grilled salmon for extra omega-3s.
- **Vegetarian:** replace the tuna with an extra egg and chickpeas.
- **Packing:** keep the dressing separate and add just before eating.
- **Mercury note:** canned light tuna (skipjack) is lower in mercury than albacore; mixing up your fish choices through the week is a good idea.

WHY IT HELPS

Tuna and eggs provide high-quality protein and vitamin B12, white beans add fiber, and the vegetables provide vitamin C and folate. Olive oil and olives contribute healthy fats.

Quinoa Tabbouleh with Chickpeas

GF · DF · VG · MAKE-AHEAD

SERVES	PREP	COOK	KEEPS
4	20 min	15 min	4 days

Real tabbouleh is more herb than grain, and this version keeps that spirit: armfuls of parsley and mint, juicy tomatoes and cucumber, and plenty of lemon. Quinoa stands in for bulgur, so it's naturally gluten-free, and chickpeas make it a complete lunch. It's the kind of salad that makes you feel brighter just looking at it.

Ingredients

- 3/4 cup (130 g) quinoa, rinsed
- 1 1/2 cups (360 ml) water
- 1 can (15 oz / 400 g) chickpeas, drained and rinsed
- 1 large bunch (60 g) fresh flat-leaf parsley, finely chopped
- 1 small bunch (15 g) fresh mint, finely chopped
- 1 1/2 cups (225 g) cherry tomatoes, quartered
- 1/2 cucumber, finely diced
- 3 scallions, thinly sliced
- 1/4 cup (60 ml) olive oil
- Juice of 1 1/2 lemons
- 1/2 tsp ground cumin
- Salt and black pepper
- 2 tbsp pomegranate seeds or toasted pine nuts, to serve (optional)

Method

1. Put the quinoa and water in a saucepan with a pinch of salt. Bring to a boil, then cover and simmer gently for 12 to 15 minutes, until the water is absorbed. Remove from the heat, leave covered for 5 minutes, then fluff with a fork and spread on a plate to cool.
2. In a large bowl, combine the cooled quinoa, chickpeas, parsley, mint, tomatoes, cucumber and scallions.
3. Whisk together the olive oil, lemon juice, cumin, salt and pepper. Pour over the salad and toss well.
4. Taste and add more lemon or salt if needed. Top with pomegranate seeds or pine nuts if you like.

SWAPS & VARIATIONS

- **More protein:** add crumbled feta (not vegan), grilled chicken, baked falafel or a side of hummus.
- **Grain swap:** if you don't need it gluten-free, use bulgur wheat for a traditional tabbouleh.
- **Make it a wrap:** stuff into a whole-grain pita with hummus.

WHY IT HELPS

Quinoa and chickpeas together provide plant protein and fiber. Parsley is rich in vitamin C, vitamin K and folate, and the olive oil and lemon dressing adds healthy fats and fresh flavor.

Miso-Glazed Tofu Buddha Bowl

DF · VG · GF OPTION

SERVES	PREP	COOK	KEEPS
2	15 min	25 min	3 days

Golden, sticky cubes of tofu with a sweet-salty miso glaze, over nutty brown rice with crisp vegetables and a creamy ginger dressing. It's the bowl I crave when I want something that feels nourishing and a little indulgent at the same time. Miso, a fermented soybean paste, brings deep savory flavor and live cultures.

Ingredients

- 1 block (14 oz / 400 g) extra-firm tofu, pressed and cut into cubes
- 1 tbsp white miso paste
- 1 tbsp low-sodium soy sauce or tamari
- 1 tsp maple syrup
- 1 tsp toasted sesame oil
- 1 cup (190 g) cooked brown rice per person (or quinoa)
- 1 cup (90 g) broccoli florets
- 1 carrot, grated or ribboned
- 1/2 cup (75 g) shelled edamame, thawed
- 1/2 avocado, sliced
- 1 tsp toasted sesame seeds

For the dressing

- 1 tbsp white miso paste
- 1 tbsp tahini
- 1 tbsp rice vinegar
- 1 tsp grated fresh ginger
- 2 tbsp warm water

Method

1. Heat the oven to 400°F (200°C) and line a baking tray with parchment.
2. Whisk together the miso, soy sauce, maple syrup and sesame oil. Toss the tofu in the mixture until coated.
3. Spread the tofu on the tray and bake for 20 to 25 minutes, turning halfway, until golden and slightly crisp.

4. Meanwhile, steam or microwave the broccoli for 3 to 4 minutes until just tender.
5. Whisk the dressing ingredients together until smooth.
6. Divide the rice between bowls and top with the tofu, broccoli, carrot, edamame and avocado. Drizzle with the dressing and sprinkle with sesame seeds.

SWAPS & VARIATIONS

- **Gluten-free:** use tamari and a miso labeled gluten-free (some miso contains barley).
- **Quicker:** pan-fry the tofu in a nonstick pan for 8 to 10 minutes, adding the glaze for the last minute.
- **Swap the protein:** use salmon fillets brushed with the same glaze (bake for 12 to 15 minutes).
- **Miso tip:** add miso to the dressing without heating, to keep more of its live cultures.

WHY IT HELPS

Tofu, edamame and miso provide three different forms of soy, with protein and isoflavones. Brown rice adds fiber and B vitamins, and broccoli brings calcium, vitamin C and fiber.

Lemon Herb Chicken Grain Bowl

DF · GF OPTION · MAKE-AHEAD

SERVES	PREP	COOK	KEEPS
4	15 min	25 min	4 days

The meal-prep hero of this book. One batch of garlicky, lemon-marinated chicken and a pot of grains gives you four lunches that taste like they came from a Mediterranean café, and any leftover chicken goes straight into wraps and soups later in the week. If you only try one make-ahead lunch, make it this one.

Ingredients

- 1 1/4 lb (570 g) boneless, skinless chicken breasts or thighs
- 2 tbsp olive oil
- Finely grated zest and juice of 1 lemon
- 2 garlic cloves, crushed
- 1 tsp dried oregano
- 1/2 tsp smoked paprika
- Salt and black pepper
- 3 cups (about 500 g) cooked farro, freekeh or quinoa
- 2 cups (60 g) baby spinach or mixed leaves
- 1 cup (150 g) cherry tomatoes, halved
- 1/2 cucumber, diced
- 1/2 cup (120 g) hummus
- 1/4 cup (35 g) pitted olives (optional)
- Fresh parsley, to serve

Method

1. In a bowl, mix the olive oil, lemon zest and juice, garlic, oregano, paprika, salt and pepper. Add the chicken and turn to coat. Marinate for at least 15 minutes, or overnight in the fridge.
2. Heat a grill pan or frying pan over medium-high heat. Cook the chicken for 5 to 7 minutes per side (breasts) or 6 to 8 minutes per side (thighs), until cooked through, with no pink meat, and the juices run clear. A meat thermometer should read 165°F (74°C).
3. Rest for 5 minutes, then slice.

4. Divide the grains between four containers or bowls. Top with spinach, tomatoes, cucumber, chicken, a spoonful of hummus and olives. Finish with parsley and an extra squeeze of lemon.

SWAPS & VARIATIONS

- **Gluten-free:** use quinoa or brown rice instead of farro or freekeh.
- **Vegetarian:** replace the chicken with chickpeas roasted in the same marinade at 400°F (200°C) for 25 minutes, or halloumi slices (not dairy-free).
- **Cook once, eat three times:** cook extra chicken to tuck into a whole-grain wrap with hummus and salad, or stir into Tuscan White Bean & Kale Soup.

WHY IT HELPS

Chicken provides lean, high-quality protein to protect muscle. Whole grains add fiber and B vitamins, hummus contributes plant protein and healthy fats, and the vegetables supply vitamins and antioxidants.

Spiced Red Lentil Dal with Spinach

GF · DF · VG · MAKE-AHEAD

SERVES	PREP	COOK	KEEPS
4	10 min	30 min	5 days

Dal is the dish so many South Asian families turn to at the end of a long day, and once you've made it you'll understand why. Red lentils melt into a thick, golden, gently spiced stew in about 25 minutes, no soaking needed, and a sizzle of popped mustard and cumin seeds on top makes it sing. It's one of the cheapest, most nourishing meals you can cook.

Ingredients

- 1 tbsp olive oil or ghee
- 1 onion, finely chopped
- 3 garlic cloves, crushed
- A thumb-sized piece (1 in / 2.5 cm) of fresh ginger, grated
- 1 tsp ground cumin
- 1 tsp ground coriander
- 1 tsp ground turmeric
- 1/2 tsp garam masala
- 1 cup (200 g) red lentils, rinsed well
- 1 can (14 oz / 400 g) chopped tomatoes
- 3 cups (720 ml) low-sodium vegetable stock or water
- 3 large handfuls (90 g) baby spinach
- Juice of 1/2 lemon
- Salt, to taste
- Fresh cilantro and plain yogurt, to serve
- Cooked brown rice or whole-grain flatbread, to serve

Optional tempering (tadka)

- 1 tsp olive oil
- 1/2 tsp cumin seeds
- 1/2 tsp mustard seeds

Method

1. Heat the oil in a large saucepan over medium heat. Add the onion and cook for 6 to 8 minutes until soft and golden.

2. Add the garlic and ginger and cook for 1 minute. Stir in the cumin, coriander, turmeric and garam masala and cook for 30 seconds, until fragrant.
3. Add the lentils, tomatoes and stock. Bring to a boil, then reduce the heat and simmer, partly covered, for 20 to 25 minutes, stirring occasionally, until the lentils are completely soft and the dal is thick and creamy. Add a splash more water if it becomes too thick.
4. Stir in the spinach until wilted. Add the lemon juice and season with salt.
5. For the tadka (optional): heat the oil in a small pan, add the seeds and cook for 30 to 60 seconds until they pop. Pour over the dal.
6. Serve with cilantro, a spoonful of yogurt and brown rice or flatbread.

SWAPS & VARIATIONS

- **Vegan:** use oil instead of ghee and serve with soy or coconut yogurt.
- **More protein:** stir in cubes of paneer (not vegan) or tofu, or serve with a boiled egg.
- **Creamier:** replace 1 cup of the stock with light coconut milk.
- **Cook once, eat three times:** serve with rice for dinner, pack for lunch, then thin with stock for a quick soup.
- **Freezing:** freezes well for up to 3 months.

WHY IT HELPS

Red lentils provide plant protein, fiber, iron and folate. Spinach adds more iron and magnesium, and the tomatoes and lemon provide vitamin C, which helps your body absorb the iron. Turmeric, ginger and cumin add flavor and anti-inflammatory compounds.

Smashed Chickpea Salad Sandwich

DF · VG · GF OPTION · 15 MIN

SERVES	PREP	COOK	KEEPS
2	15 min	None	3 days

Think of everything you love about a deli egg-salad sandwich, creamy, tangy and crunchy, then make it from a can of chickpeas in ten minutes. Celery and red onion bring the crunch, capers and lemon the zing. It's my go-to when the fridge is nearly empty and I still want a lunch I'll look forward to.

Ingredients

- 1 can (15 oz / 400 g) chickpeas, drained and rinsed
- 2 tbsp tahini or vegan mayonnaise
- 1 tsp Dijon mustard
- Juice of 1/2 lemon
- 1 celery stalk, finely diced
- 1/4 red onion, finely diced
- 1 tbsp capers or chopped pickles
- 1 tbsp chopped fresh dill or parsley
- Salt and black pepper
- 4 slices whole-grain or seeded bread
- 1 large handful lettuce or arugula
- 1 tomato, sliced

Method

1. Put the chickpeas in a bowl and mash with a fork or potato masher until mostly broken down, leaving some texture.
2. Add the tahini, mustard and lemon juice and mix well.
3. Stir in the celery, red onion, capers and herbs. Season with salt and pepper.
4. Spread the filling over two slices of bread, top with lettuce and tomato, and close the sandwiches.

SWAPS & VARIATIONS

- **Gluten-free:** use gluten-free bread or serve in lettuce cups or with rice cakes.
- **Not vegan:** use Greek yogurt or regular mayonnaise instead of tahini.
- **Flavor twists:** add curry powder and raisins, or chopped sun-dried tomatoes and basil.
- **Packing:** pack the filling separately and assemble at lunchtime so the bread doesn't go soggy.

WHY IT HELPS

Chickpeas provide plant protein, fiber and small amounts of isoflavones. Tahini adds calcium and healthy fats, and whole-grain bread contributes extra fiber and B vitamins.